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When They Forget Your Name

The Marriage That Survives Dementia

How Couples 50+ Hold On When One Starts Letting Go

A 4-Phase Framework for the Spouse Who Remembers
by Sanjay Shharma

Author of The Care Collection

Copyright Page

When They Forget Your Name: The Marriage That Survives Dementia

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This is a work of nonfiction. All names, identifying details, and composite case studies have been changed to protect the privacy of the individuals involved. The case studies in this book are composites drawn from research, common patterns, and the lived experience of caregivers and spouses. Any resemblance to specific individuals is coincidental.

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Disclaimer: This book is not a substitute for medical, psychiatric, psychological, or legal advice. Dementia is a medical condition that requires professional diagnosis, treatment, and ongoing care. The author is not a licensed medical professional. The content of this book is for informational and educational purposes only. If you or your spouse is experiencing symptoms of cognitive decline, please consult a qualified healthcare provider. If you are in crisis as a caregiver — feeling overwhelmed, depressed, or

considering harming yourself or your spouse — please reach out to a crisis line or mental health professional immediately. The resources section at the back of this book includes crisis contacts in multiple countries.

Crisis Resources (US):

- National Suicide Prevention Lifeline: 988
- Alzheimer's Association Helpline:
1-800-272-3900
- Crisis Text Line: Text HOME to 741741

Crisis Resources (UK):

- Samaritans: 116 123
- Alzheimer's Society: 0300 222 1122

Crisis Resources (India):

- iCall: 9152987821
- Alzheimer's and Related Disorders Society of India: 9846311698

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Dedication

For the spouse who still remembers.

For the person who wakes up every morning next to someone who no longer knows their name — and chooses to love them anyway.

For the caregiver who is slowly losing the person they married, while the rest of the world moves on.

For the husband who holds his wife's hand even though she pulls away.

For the wife who tells the same story three times in an hour and is listened to with the same patience each time.

For the marriages that survive this — not because they are perfect, but because they refuse to give up.

This book is for you.

And for the silent army of spouses who walk this road alone, with no map, no script, and no permission to grieve the living.

You are not invisible. You are not alone. And the marriage you are keeping alive — in whatever form it now takes — is not a failure.

It is one of the bravest things a human being can do.

A Note from the Author

I have been on this side of the equation. Not dementia — cancer. But the journey of watching the person you love disappear piece by piece, of being the one who remembers when they don't, of being the caregiver while the rest of the world moves on, of grieving someone who is still in the same room — that journey I know.

My wife was diagnosed with cancer. I was her primary caregiver. I learned, in those years, what it means to hold the marriage together when the person you married can no longer hold it with you. I learned that love does not end when memory does. I learned that the caregiver's grief is real, even when the death has not yet come. I learned that the marriage you have after the diagnosis is a different marriage from the one you had before — and that the new marriage, if you let it, can still be a real one.

This book is for the spouse who is living it. Not a memoir. A map.

The patterns in this book come from research, from common experience, and from the lives of people I have spoken with — both as a coach and as a fellow traveler in the long goodbye. The case studies are composites. The frameworks are drawn from the work of many. The voice is mine.

You are not the disease. You are not the caregiver. You are not the diagnosis.

You are the person who still says their name — even when they cannot say yours.

That person deserves a book.

This is that book.

— S. Shharma

Quick Start

If you have just opened this book and you do not have time to read it cover to cover right now, start here.

You are in one of four phases. Find yours:

Phase 1: The Day You Notice — You are starting to see things. The keys. The dates. The conversations. You are not sure yet if this is aging or something more. Read Chapters 1-4.

Phase 2: The Diagnosis Year — You have a name for it now. Alzheimer's, dementia, MCI. The first year after a diagnosis is its own country. You are figuring out who you are in it. Read Chapters 5-8.

Phase 3: The Long Middle — The slow goodbye that doesn't end. Years, sometimes. The person is still here, but the person you married is fading. You are tired. You are angry. You are alone in a way no one can see. Read Chapters 9-13.

Phase 4: The Letting Go — The end is visible now, in the room, in the body, in the breath. You are about to lose them, or you already have. You are figuring out who you are without them. Read Chapters 14-16 and the Epilogue.

If you are in crisis right now — go to Bonus Toolkit IV: The Caregiver's First Aid Kit. It has 7 things you can do in the next 24 hours.

If you need permission to grieve — go to Chapter 15: The Grief That Has No Name.

If you need scripts — go to Bonus Toolkit II: The 12 Conversations.

If you need to know when to place your spouse in care — go to Bonus Toolkit III: The Decision Tree.

If you are reading this for someone else — a friend, an adult child, a sibling — start with Chapter 6: Telling the Children, the Friends, the World. Then read the rest.

One thing this book is not: a clinical guide. The medical, pharmacological, and caregiving-research literature is vast and important. This book is the marriage side. The part no one writes about. The part that happens between the doctor appointments and the daily routines. The part where two people who promised to love each other are trying to figure out what love means when one of them is leaving.

Welcome. Take a breath. Let's begin.

Introduction: The Spouse Who Remembers

There is a moment, somewhere in the long slow course of a marriage, when one partner becomes the keeper of the other's life. It does not happen at the wedding. It does not happen at the birth of the children. It does not happen at the silver anniversary. It happens, often without warning, at the moment the other person begins to forget.

This book is about that moment. And the years that follow it.

The diagnosis is Alzheimer's. Or dementia — frontotemporal, Lewy body, vascular. Or it is a long slide that never gets a name, just a slow withdrawing of the person you knew. Whatever the cause, the shape is similar: the marriage you had is changing into a marriage you did not ask for, and the person you married is changing into someone you must learn to love in a new way.

You are reading this because you are the spouse who remembers.

You are the one who knows the stories. The names of the children. The address they lived at for thirty years. The first time they laughed at a joke you told. The way they take their coffee. The song that was playing when you first met. You hold all of it now — alone. The whole archive of your shared life, in one head, in one heart.

This book is not about how to fix your spouse. There is no fixing. There is no reversing. There is no cure that is coming in time to save your marriage. That is the first hard truth.

This book is about the other half of the diagnosis. The half the doctors don't talk about, the half the medical books skip over, the half that lives in the bedroom at 11pm when they are asleep and you are not. The half that is the marriage itself. The relationship. The "us" that you are still trying to be, even when one of you can no longer remember what "us" means.

There are roughly 55 million people in the world living with dementia today. By 2050, that number is expected to nearly triple. Most of them are over 65. Most of them are married. And most of them have a spouse — usually older themselves — who is doing the remembering for both of them.

That spouse is who I am writing for.

I am not a neurologist. I am not a geriatrician. I am not a researcher. I am a businessman, a writer, and someone who has been on the other side of a long goodbye. I watched my wife disappear. Not from dementia — from cancer. But the shape of the journey was the same. The slow erosion. The role inversion. The loneliness. The grief that has no name because the person is still alive. The marriage that has to keep being a marriage even when one of you can no longer hold up your half of it.

What I learned in those years is this: the marriage does not have to end. It changes. The vows you made — for better or worse, in sickness and in health — were not a contract that expires when memory does. They were a practice. And a practice, like any practice, can be done in a new way.

This book is that new way.

It is not a clinical manual. It is not a research review. It is not a memoir. It is a map for the spouse who is still here, drawn by someone who has walked a similar

road and come out the other side changed, and is now writing down what he wishes he had known.

You will find four phases in this book — the four stages of the journey of the spouse who remembers. You will find case studies, composite couples who have lived these phases, drawn from research and common experience. You will find scripts, tools, decision trees, and rituals. You will find, if you read carefully, permission. Permission to grieve the person who is still in the room. Permission to be angry. Permission to ask for help. Permission to put them in care, or to keep them home. Permission to survive.

You are not invisible. You are not alone. And the marriage you are keeping alive — in whatever form it now takes — is not a small thing.

It is one of the bravest things a human being can do.

Let us begin.

Chapter 1: The Forgetting That Isn't Normal

The first sign of dementia is not forgetting a name, or losing a set of keys, or walking into a room and not remembering why. Those things happen to everyone, especially as we get older. The first sign is a pattern of forgetting that is different from the way your spouse has always forgotten. It is qualitatively different. It is the kind of forgetting that is new to them — to the person they have always been.

It is the kind of forgetting that makes you, the spouse, stop and wonder.

Margaret was sixty-three when she started noticing. Her husband Robert, a retired civil engineer, had always been the organized one. He kept the family calendar. He remembered birthdays. He could recite the names of every person they had ever met at church, in order, going back thirty years. Margaret was the mess. She lost her glasses three times a day. She forgot why she had walked into rooms. She wrote herself notes on the back of receipts.

So when Robert started to do the things Margaret had always done — lose his glasses, forget appointments, lose the thread of a conversation — Margaret did not worry. She teased him. You've finally joined me, she said. Welcome to the club.

It took her eighteen months to realize the difference.

Margaret was forgetting the way she had always forgotten — randomly, without pattern, in clusters. Robert was forgetting in a progression. The glasses first. Then the appointments. Then the conversations. Then the names. Then the memories that had been his

identity — his career, his children, his mother.

The pattern was not random. It was a slope. And it was going down.

This is the shape of the early forgetting. It is not the forgetting itself that is the warning. It is the direction of the forgetting. The first sign of dementia is the moment you notice that the forgetting is not random but progressive. That the person you have known for thirty or forty years is starting to lose the things they used to know how to keep.

There are common early signs. Not every person with dementia will have all of them, and not every sign means dementia — many of these can be caused by medication, depression, sleep apnea, vitamin deficiencies, or normal aging. But the combination of several of them, progressing over time, is the pattern to watch for.

The first signs:

1. Short-term memory loss that disrupts daily life. Your spouse asks the same question three times in an hour. They forget what you told them at breakfast by lunch. They lose track of what day it is, not in the casual way of someone who never paid attention to dates, but in the way of someone who used to remember perfectly and now cannot.
2. Difficulty with familiar tasks. Your spouse has made the same coffee every morning for thirty years. Now they put the kettle on the stove and the coffee in the fridge. They cannot remember how to set the microwave. They get lost driving to the grocery store they have shopped at for fifteen years.
3. Language problems. Your spouse stops in the middle of a sentence and cannot find the word. They

substitute wrong words — "the thing that holds the food" for "refrigerator." They call the grandchildren by the wrong names. They lose the thread of their own stories.

4. Disorientation to time and place. Your spouse forgets where they are, not in the transient way of a tourist in an unfamiliar city, but in the way of someone who is in their own living room and does not know it.

5. Poor or decreased judgment. Your spouse makes unusual decisions with money, or hygiene, or safety. They give money to a telemarketer. They wear the same clothes for a week. They leave the stove on.

6. Misplacing things and losing the ability to retrace steps. Your spouse puts the car keys in the freezer. They cannot retrace where they have been. They accuse you or others of moving their things.

7. Changes in mood or personality. Your spouse becomes suspicious, anxious, fearful, or withdrawn. They get angry in ways they never used to. They are suddenly afraid of things that never bothered them.

8. Loss of initiative. Your spouse stops doing the things they used to love. The hobbies. The reading. The garden. The phone calls to friends. They sit in a chair and stare.

If you are reading this and several of these patterns look familiar, and they have been progressing for six months or more, the next step is a medical evaluation. Not a self-diagnosis. Not a Google search. A neurologist, geriatrician, or memory clinic. There are reversible causes of memory loss — medications, vitamin B12 deficiency, thyroid issues, depression, sleep disorders, infections. These need to be ruled out first.

If the cause is dementia, you will need a diagnosis. You will need a name for what is happening. Not because the name will change it, but because the name will allow you to begin to plan.

But before the doctor, before the diagnosis, there is a longer, harder, more private process. The process of noticing.

The first weeks of noticing are the loneliest weeks of the whole journey. You cannot say anything yet. The forgetting is not bad enough to be undeniable. It is not consistent enough to bring to a doctor. You are seeing things, and the people around you are not seeing them. Your adult children tell you that dad is just getting older. Your friends tell you that mom is fine. Your spouse tells you they are fine.

You begin to wonder if you are imagining it. You begin to wonder if you are being unfair. You begin to wonder if the forgetting is not in your spouse, but in you — in your expectations, in your anxiety, in your need for control.

This is the trap of the first stage. The trap is not that you are imagining it. The trap is that you will silence yourself in order to be a good spouse, a kind spouse, a fair spouse. You will not want to make a fuss. You will not want to be the spouse who panics. You will not want to take away their dignity, their autonomy, their personhood.

But the alternative is worse. The alternative is to wait. The alternative is to let the slope continue. The alternative is to arrive at the doctor's office eighteen months from now, in crisis, when the forgetting has gone past the point of early intervention.

If you are noticing, you are not imagining. The pattern is real. The slope is real. The disease is real.

The most loving thing you can do, in this first stage, is to be the one who says, I have noticed something, and I think we should look into it together.

Not I have noticed something is wrong with you.

Not You need to see a doctor.

Together. As a couple. As a partnership that is facing a new challenge, the way partnerships do.

That is the conversation of Chapter 3. But first, Chapter 2: why you, the spouse, are the one who sees it first.

Chapter 1 at a Glance

Key takeaway: The first sign of dementia is not forgetting. It is the pattern of forgetting. The slope. The direction. The difference from the way your spouse has always been.

5 Things to Remember:

1. Random forgetting is normal. Progressive forgetting is the warning sign.
2. The first signs are usually short-term memory loss, familiar tasks, language, orientation, judgment.
3. The first weeks of noticing are the loneliest. You are seeing what no one else sees.
4. The trap is silencing yourself to be a "good spouse." That silence can cost you eighteen months of early intervention.
5. The first move is to bring it up — together, as a couple, before the slope goes too far.

Your Scorecard: Rate yourself on a scale of 1-5 for each:

- I have noticed a pattern (not just incidents) of memory loss in my spouse.
- The pattern has been going on for six months or longer.
- I have been able to identify 3+ of the early signs.
- I have not yet brought it up with my spouse or a doctor.

If your total is 12 or higher, the next chapter — and the next conversation — cannot wait.

Key Terms to Know:

- **MCI (Mild Cognitive Impairment):** A stage between normal aging and dementia, where memory problems are noticeable but not severe enough to interfere with daily life. About 10-15% of people with MCI progress to dementia each year.
- **Dementia:** An umbrella term for a decline in mental ability severe enough to interfere with daily life. Alzheimer's is the most common type (60-80% of cases).
- **Alzheimer's Disease:** A specific type of dementia, characterized by plaques and tangles in the brain, that progresses through seven stages over an average of 4-8 years after diagnosis.
- **Reversible causes:** Memory loss that looks like dementia but is caused by something treatable — vitamin B12 deficiency, thyroid disease, depression, sleep apnea, medication side effects. Always rule out before assuming.

Reflection Questions:

1. When did you first notice the pattern, and what did you do about it?

2. Have you been silencing yourself to be a "good spouse"? What did that cost you?

3. What would it look like to bring this up "together" with your spouse, as a partnership facing a challenge?

Action for This Week:

If you have been noticing, this is the week to do one of three things:

- Have the conversation with your spouse. (See Chapter 3 for scripts.)
- Schedule a doctor's appointment. (Even a primary care visit to rule out reversible causes is a start.)
- Write down the patterns you have seen. (Dates, examples, what was different from how they used to be.) The written record will matter when you get to the doctor's office.

Chapter 2: The Spouse Who Knows First

There is a particular loneliness that belongs to the person who is the first to notice. It is not the loneliness of being alone in a room. It is the loneliness of being alone in a knowing. You have seen something that no one else has seen yet. The doctor has not seen it. The children have not seen it. Your friends have not seen it. Your spouse, in many cases, has not seen it. You are carrying a piece of knowledge that you cannot yet share, and the weight of that knowledge is changing you.

Why does the spouse see it first?

Because the spouse is the only other person in the world who has lived with this person for thirty, forty, fifty years, day in and day out, in the small hours and the ordinary hours. The spouse knows the rhythms. The patterns. The way the other person takes their tea. The way they tell a story. The way they load the dishwasher. The way they sigh when they are tired. The way they walk into a room. The way they laugh.

When something shifts, the spouse feels it the way a musician feels a note go out of tune. Not because the musician is a perfectionist. Because the musician's ear knows what in tune sounds like, and out of tune is unmistakable.

The adult children do not see it because they are not there every day. They see their parent on Sundays, or on holidays, or for the occasional weekend. By then, the parent has had time to gather themselves, to perform the social self. The forgetting that happens at home, in the unguarded moments — the word lost in

the middle of a sentence, the question asked for the third time, the coffee put in the cupboard — does not happen in front of the children. The parent, even in early dementia, often has a residual social awareness that allows them to cover.

The friends do not see it because they see the same limited version. They do not have the baseline. They cannot tell the difference between the person they have known socially for ten years and the person they have known socially for ten years who is starting to slip. Without the intimate knowledge of the daily rhythms, the small slips are invisible.

The doctor does not see it because the doctor has fifteen minutes, and the person sitting in the examination room is often performing. They have been told, in many subtle ways, that they are there to demonstrate competence. They will often over-perform in the doctor's office. The doctor's office is a stage, and the patient knows the lines.

The spouse sees it because the spouse is the only audience for the unscripted performance. The spouse is the one who is there at 11pm when the script has run out. The spouse is the one who is there in the morning, before the social self has reassembled. The spouse is the one who knows that something is different, even when the something is small.

And then comes the loneliness of the knowing.

The loneliness is not about other people failing to see it. The loneliness is about the changes it brings in you.

You begin to watch. You begin to evaluate. You begin to monitor. Every sentence they say, you are parsing. Every action they take, you are checking against the version of them that lives in your memory. You are no longer simply living with your spouse. You are

observing them. You are taking measurements. You are keeping a record.

This is a profound change in the marriage. You are still on the same side of the bed, eating at the same table, but the relationship has shifted from being with to being on watch over. The marriage has become, in a small but unmistakable way, a clinical relationship. You are the doctor, in the home. You are the monitor. You are the one who notices the symptom.

And the person you are monitoring can feel it. Even in early dementia, even before they have any words for what is happening, your spouse can feel the change in you. They can feel that you are watching. They can feel that something has shifted. They may not know what it is. But they can feel that you are no longer just their partner. You have become something else. You have become the one who is looking for signs.

This is the first damage of the diagnosis. Not the memory loss itself. The change in the relationship that the memory loss brings, even before it has a name.

And the second damage is the silence.

You cannot tell anyone. You cannot tell your spouse — because they will deny it, or become angry, or become terrified, and you are not yet ready to manage any of those reactions. You cannot tell your children — because they will not believe you, or they will panic, or they will treat your spouse differently, and you are not ready for that. You cannot tell your friends — because you are not yet sure, and saying it out loud will make it real in a way you are not yet ready for.

So you hold it. Alone.

You hold it while you make dinner. You hold it while you go to bed. You hold it while you make love. You

hold it while you go to the grocery store, watching them try to remember why they came. You hold it while they tell the same story they told yesterday, in the same words, to the same friend, on the same call.

And the loneliness grows. Because the marriage that was supposed to be the place where you share everything has become the place where you cannot share the most important thing.

There is a way through this. The way through is not to keep holding it. The way through is to begin to find the people who can hold it with you.

The first person to tell is your doctor. Not the doctor of your spouse — your own doctor. Because you are going to need to be in good shape for the long road ahead, and the long road begins now.

The second person to tell is a close friend. Not someone who will panic, not someone who will gossip, but someone who has the maturity to hear what you are seeing and to hold it without judgment. The friend does not need to do anything. The friend just needs to know. So that you can say the things you cannot say to your spouse. So that you can cry in a room with someone who is not your spouse. So that you are not alone in the knowing.

The third person to tell is one of your adult children. Pick the one who is most likely to be calm. Pick the one who is most likely to listen before reacting. Tell them what you have been seeing. Tell them you are not yet sure. Tell them you are not yet ready for the whole family to know. But tell them. Because the load is too heavy to carry alone, and the silence is starting to make you sick.

And the fourth person — eventually, when you are ready, when the slope is undeniable, when the doctor

is involved — is your spouse. That conversation is in Chapter 3.

For now, the work of this chapter is to name what you have been doing. You have been the first to see. You have been carrying the knowledge alone. You have been the spouse who remembers, in a marriage that is starting to change in ways you cannot yet name.

You are not imagining it. You are not being unfair. You are not being a bad spouse by noticing. You are being the spouse — the one who has the intimate knowledge, the daily baseline, the pattern recognition that no one else has.

The next chapters are about what to do with that knowing. The first step is to make sure you are supported. The second is to begin the medical conversation. The third is to bring your spouse into the conversation in a way that preserves their dignity, your partnership, and the marriage.

You cannot do any of those things if you are carrying the knowledge alone. So start now. Tell one person. Today.

Chapter 2 at a Glance

Key takeaway: The spouse sees it first because the spouse is the only one with the daily baseline. But the seeing is lonely, and the loneliness is damaging. The cure is to begin sharing the load.

5 Things to Remember:

1. The spouse sees it first because of the intimate baseline — the daily rhythms no one else knows.
2. The first damage of the diagnosis is the change in the relationship — from being with to being on watch over.

3. The second damage is the silence. You cannot tell anyone, and the holding is making you sick.
4. The cure is to start telling — your own doctor, a close friend, a calm adult child, and eventually your spouse.
5. You are not imagining it. You are not being unfair. You are being the spouse.

Your Scorecard: Rate yourself 1-5:

- I have been noticing patterns for six months or more.
- I have been carrying the knowledge largely alone.
- I have begun to feel like I am "monitoring" my spouse rather than just being with them.
- I have not yet told anyone outside the marriage.

If your total is 12 or higher, the most important thing you can do this week is tell one person.

Key Terms to Know:

- Baseline: The pattern of how someone has always been. The spouse has the baseline; no one else does.
- Covering: The way people in early dementia often perform socially, hiding the forgetting in front of others but losing the script at home.
- Anticipatory grief: The grief that begins before the loss. Begins for the spouse at this stage, often unnamed.

Reflection Questions:

1. When did you first feel the shift from "being with" to "watching over"? What did it cost you?
2. Who is the one person you could tell this week? What would that conversation look like?

3. What would it take for you to make an appointment with your own doctor to talk about the load you are carrying?

Action for This Week:

Tell one person. Pick the calmest, most mature person in your circle. Say: I need to tell you something I haven't been able to say out loud. I have been noticing things about [spouse's name] that worry me. I don't have a name for it yet, but I need to tell someone. Can you just listen? That is the entire action. Nothing else is required. You are not asking for advice. You are not asking for a solution. You are asking for the loneliness to end.

Chapter 3: The Conversation You Don't Want to Have

There is a conversation that every spouse in this position eventually has to have. It is the conversation where you say, I have noticed something, and I think we should look into it together. It is a conversation you will rehearse in your head a hundred times. It is a conversation you will delay for months. It is a conversation that, when you finally have it, will not go the way you rehearsed it.

This chapter is about how to have that conversation.

The first thing to know is that there is no perfect way. There is no script that will get you to a good outcome. There is no tone, no wording, no setting that will make your spouse receive the news with grace, with relief, with the calm acceptance you imagine in your head. Real people, faced with the possibility of dementia, react in a hundred different ways. They deny. They get angry. They cry. They laugh it off. They accuse you of plotting against them. They shut down. They go quiet. They blame themselves. They blame you. They retreat into a childlike state that breaks your heart.

The second thing to know is that the conversation has to happen. There is no version of this that does not include a conversation. The slope is real. The disease is real. The medical evaluation is real. The earlier the diagnosis, the more options you have, the more time you have to plan, the more agency you retain. The longer you wait, the more the slope takes from you.

So the question is not whether to have the conversation, but how.

Choose the moment carefully.

Not in an argument. Not in a crisis. Not after they have just lost their way home. Not in front of the children. Not at a holiday gathering. Not when either of you is tired, hungry, or stressed.

The best moment is a quiet one. A morning when the coffee is on and the day is open. A weekend afternoon when the house is still. A walk together, in a place you both know well, with no urgency to be anywhere. The setting should signal: this is not an emergency, this is a conversation between two people who love each other about something important.

Lead with love, not with symptoms.

The instinct is to begin with the list. You forgot our anniversary. You asked the same question four times. You put the car keys in the fridge. But that list, however true, will land as an attack. It will land as: You are failing. You are doing these things wrong. I have been keeping track of your failures.

Instead, lead with where you are. I have been thinking about us. About our future. About getting older. And there is something I want to talk about, not because I think something is wrong, but because I want us to be ready for whatever comes.

This is not dishonest. It is a reframe. You are not keeping a list of failures. You are concerned about the future. That is the truth. The list is the evidence, but the conversation is about the future.

Make it a we, not a you.

I have noticed some changes in both of us. No — that is dishonest, and they will see through it. But I have noticed some changes in you, and I want us to look into it together is closer to the truth.

Or, more simply: I love you, and there are some things I have been noticing, and I think we should talk to a doctor about them. Not because anything is wrong — because I want to make sure we are taking care of us.

The framing matters. You are not accusing. You are not diagnosing. You are not the doctor. You are the spouse. You are saying: I love you, and I want us to be okay. There is a small thing, and I want to get it checked, so we can stop worrying about it if it's nothing, or get help early if it's something.

Be ready for the reaction.

Your spouse may:

- Deny. I'm fine. There's nothing wrong with me. You're the one with the problem. Do not argue. Do not list examples. Say: I hear you. I might be wrong. Let's get it checked so we can both stop worrying.
- Get angry. How dare you. After everything I've done for you. Do not defend. Do not apologize for noticing. Say: I understand this is hard to hear. I'm not trying to upset you. I'm trying to take care of us.
- Cry. I knew it. I knew something was wrong. I was too scared to say it. This is a gift. They were already afraid. You have given them permission to say it. Hold them. Let them cry. Then, when they are ready, make the appointment together.
- Make a joke. Well, I've been losing my mind for years. At least now I have an excuse. This is not denial. This is humor as a defense. Do not laugh. Do not push. Say: Let's just get it checked, so we can put your mind at rest.
- Shut down. They go quiet. They leave the room. They do not want to talk about it. Give them space.

Bring it up again in a few days, gently, with a specific next step: I made an appointment with Dr. [name] for next Tuesday. I'll come with you. It will just be a conversation.

- Accuse you. You're trying to take my car away. You're trying to put me in a home. You're plotting with the children. This is the fear speaking. The fear of losing autonomy. The fear of being put away. Address the fear directly: I am not trying to take anything away from you. I am trying to add things to our life. I want us to grow old together, and I want to make sure we are ready.

If they refuse to go to the doctor.

You cannot force a competent adult to see a doctor. You cannot trick them. You cannot ambush them. What you can do is:

- Ask your own doctor for advice. The doctor cannot see your spouse without consent, but the doctor can give you strategies.
- Make the appointment for yourself. Say: I'm going for my annual physical. Will you come with me? I'd love the company. Once at the doctor's office, the doctor can offer to do a quick memory screen, framed as a routine part of the visit.
- Call a geriatric care manager. These professionals specialize in exactly this situation. They can advise on how to get someone to a doctor who refuses.
- Be patient. The denial will not last forever. The slope is real. The next incident will come. Each incident is another opportunity to bring it up, gently, with love, as a couple.

What the conversation is not.

It is not a one-time event. It is the first of many. There will be the conversation about getting a second opinion. The conversation about telling the children. The conversation about stopping driving. The conversation about long-term care insurance. The conversation about a power of attorney. The conversation about moving to a smaller place. The conversation about placing them in care.

Each of these conversations is its own small grief. Each one is a small loss of the marriage as it was. Each one is a new shape of the marriage as it is becoming.

The skill you are building, in this first conversation, is the skill you will use in all the others. I love you. I have noticed something. I want us to face it together. I am not trying to take anything away. I am trying to add to what we have.

That is the conversation. That is the first move.

The next chapter is about what happens in the year that follows — the first year after the conversation, when the slope begins to be undeniable, and the marriage starts to change.

Chapter 3 at a Glance

Key takeaway: The conversation has to happen. There is no perfect way. Lead with love, frame it as we, be ready for the reaction, and know that this is the first of many conversations you will have.

5 Things to Remember:

1. The setting matters. Quiet, private, not in an argument or a crisis.
2. Lead with love and the future, not with the list of failures.

3. Make it a "we" — a couple facing a challenge, not a doctor diagnosing a patient.
4. Be ready for denial, anger, tears, jokes, shutdown, or accusations. Each is a different form of fear.
5. If they refuse, do not force. Bring it up again. Make a routine appointment. Get professional advice. Be patient.

Your Scorecard: Rate 1-5:

- I have a plan for when and how I will have this conversation.
- I have rehearsed the opening line.
- I have anticipated my spouse's likely reaction.
- I have a backup plan if they refuse.

If your total is 12 or lower, the conversation is not ready. Rehearse more. Talk to a friend or counselor first.

Key Terms to Know:

- Geriatric care manager: A professional who specializes in helping families navigate aging-related issues, including getting reluctant elders to medical appointments.
- Power of attorney (POA): A legal document that gives someone else the authority to make decisions on your behalf. Critical to set up while the person with dementia still has capacity.
- Capacity: The legal ability to make decisions. A person with dementia eventually loses capacity, which is why the POA and other legal documents need to be done early.

Reflection Questions:

1. What is the most likely reaction your spouse will have? How will you respond?

2. What is the one sentence you can rehearse as your opening?

3. If they refuse to see a doctor, what is your next step?

Action for This Week:

- Pick the moment. Pick the place. Write down your opening line.
- Identify the calmest, most supportive person in your circle. Tell them the conversation is coming, and ask if you can call them right after.
- If your spouse is in significant denial, make a one-call inquiry to a geriatric care manager in your area. Just the call. Just the information. You do not have to act on it yet.

Chapter 4: The First Year — When Nothing Has Changed and Everything Has

The first year after a diagnosis is a country with its own weather.

On the surface, nothing has changed. The house looks the same. The marriage looks the same. The daily routines are the same. Your spouse is still here, still recognizable, still doing many of the things they have always done. You are still doing what you have always done — going to work, paying the bills, calling the children, taking the dog to the vet. The world outside has not shifted.

And yet, underneath the surface, everything has changed.

There is a phrase from the dementia literature that is technically accurate and emotionally devastating: the diagnosis is for the family, not the patient. The patient, in the early stage, often has limited awareness of what is happening. Their coping mechanisms — the covering, the confabulation, the social performance — are still in place. They may genuinely not know that they have been diagnosed, in the deep sense of knowing. They may know the words, but not the weight.

You, the spouse, are the one for whom the diagnosis is real. You are the one who now carries the name — Alzheimer's, dementia, MCI — and the knowledge of what the name means. You are the one who has been told the slope. You are the one who now knows the

future, in broad outline, and who must live, day by day, in a present that is slowly becoming the future you were warned about.

This is the first year. The year of nothing changing and everything changing. The year of the slow recalibration.

The first weeks are the worst. After the diagnosis, you are shell-shocked in a way that does not match the surface calm of daily life. There is no funeral. There is no crisis. There is no visible change in your spouse. But the diagnosis has done something to you. It has changed the way you see. It has made every forgotten word, every missed appointment, every lost set of keys into a sign of the future. The slope that you were suspecting has been confirmed, and now every moment is a measurement of the slope's progress.

You may find yourself doing odd things. Counting. You start to count the times they ask the same question. You start to count the days between incidents. You start to count the bills they can no longer pay, the appointments they can no longer keep, the conversations they can no longer follow. You become, without meaning to, a meter of the disease.

You may find yourself grieving. Not for the death that has not come. For the slow death that is. For the husband or wife who is still here, but is no longer the husband or wife you married. For the future you were planning — the retirement trips, the long weekends, the quiet evenings of doing nothing together. For the conversations you were going to have, the books you were going to read together, the grandchildren you were going to spoil together.

This is anticipatory grief. The grief that begins before the loss. The grief that has no funeral, no casserole

from the neighbors, no one bringing you soup. The grief that lives in you, alone, for the marriage that is becoming something else.

You may find yourself fighting. With your spouse. With the universe. With the medical system. With yourself. The diagnosis has made you angry, and the anger has to go somewhere. It often goes to the nearest person, which is your spouse. Not because they have done anything wrong. Because they are the visible form of the disease. Because they are the daily reminder that the slope is real. Because they are the one who can no longer be the partner you need them to be, even though they are doing their best.

This is a hard thing to say out loud, and an important one: you may, in this first year, start to feel anger at your spouse. Not at the disease. At them. This is a normal response. It is not a sign of a bad marriage. It is a sign of a human being under impossible pressure. But it is something to watch, because the anger can become a wedge if you let it. The anger can become contempt, and contempt is the death of any marriage.

The first year, then, is the year of recalibrating. Recalibrating your expectations. Recalibrating your identity. Recalibrating your marriage.

Recalibrating your expectations.

You have been planning a future with a competent partner. You were going to travel. You were going to retire. You were going to grow old together in the way that couples grow old together — making decisions, sharing the load, leaning on each other.

The diagnosis means the future will be different. Not all of it. There will still be travel, of a kind. There will still be retirement, of a kind. There will still be growing old together, of a kind. But the shape of the

future is being redrawn, and the redrawing is happening in real time, in small moments, day by day.

The recalibration is the work of giving up the future you had planned and opening yourself to the future that is actually arriving. This is not a small grief. This is the grief of a life imagined, and it deserves to be honored.

Recalibrating your identity.

Before the diagnosis, you were a spouse. A partner. A companion. One half of a couple.

After the diagnosis, you are beginning to become something else. You are becoming a caregiver. You are becoming a case manager. You are becoming the one who makes the appointments, pays the bills, manages the medications, monitors the driving, monitors the cooking, monitors the hygiene, monitors the social world, monitors the medical world.

This is a profound identity shift. It happens slowly. It happens without you noticing. But by the end of the first year, you will not be the same person you were at the beginning. You will be harder. You will be more competent in ways you did not ask to be competent. You will be lonelier. You will be tired in a way that sleep does not fix.

You need to be aware of this shift, because it is the shift that breaks marriages that the disease does not break. The spouse who becomes only a caregiver stops being a spouse. The marriage becomes a clinical relationship. The love goes underground, replaced by duty.

The recalibration is to keep the love above the ground. To find small ways, every day, to be a partner, not just a caregiver. To touch your spouse's hand. To look

them in the eye. To ask them how they are, and to listen. To tell them a story from your shared past, and to laugh with them at the parts that are still funny. To do the things you have always done as a couple — the morning coffee, the evening walk, the weekly call to the children — and to do them with the awareness that these rituals are now load-bearing walls. They are holding the marriage up.

Recalibrating your marriage.

The marriage you had was a partnership of two competent adults. The marriage you have now is something else. It is a partnership in which one of you is becoming less competent, day by day, and the other is becoming more competent in the caregiving role, day by day. The asymmetry is increasing. The balance of the marriage is shifting.

The recalibration is to find a new balance. Not the old balance, where you each carried your own weight. A new balance, where you are carrying more, and they are carrying what they can. The recalibration is to make the new balance livable. To make the new balance something you can sustain for years. To make the new balance a marriage, and not just a caregiving arrangement.

This means asking for help. From the children. From friends. From neighbors. From professionals. From anyone who can take a piece of the load so that you are not the only load-bearing wall.

This means accepting the new shape of the marriage without resentment. The resentment will come. It is human for it to come. But the work of the recalibration is to feel the resentment, name it, and let it go. Not let it go because you are a saint. Let it go because the resentment is a poison that will kill the

marriage faster than the disease.

The gift of the first year.

There is a gift in the first year, though it does not feel like a gift at the time. The gift is the clarity. The diagnosis, as painful as it is, gives you the gift of knowing. You know what you are dealing with. You know the slope. You know the future, in broad outline. And that knowing, however painful, allows you to plan. To make the legal documents. To have the conversations. To make the memories while you can still make them. To say the things that need to be said.

The first year is the year of while you still can. While your spouse can still sign a document. While your spouse can still tell a story. While your spouse can still recognize a grandchild. While your spouse can still say your name.

Use this year. Do the legal work. Have the hard conversations. Make the memory book. Take the trip, if there is a trip to take. Tell them the things you have been meaning to tell them. And tell them now, while they can still hear you.

The first year is hard. It is also the most valuable year you have. Use it.

Chapter 4 at a Glance

Key takeaway: The first year after diagnosis is the year of recalibrating — your expectations, your identity, your marriage. The surface is unchanged. The undercurrent is everything.

5 Things to Remember:

1. The diagnosis is for you, the spouse, not for the patient. You are the one for whom it becomes real.

2. You may find yourself counting, grieving, and fighting. All three are normal.
3. The identity shift from spouse to caregiver is the shift that breaks marriages. Watch for it.
4. The recalibration is to keep the love above the duty. Small rituals, every day.
5. The first year is the year of "while you still can." Use it for the legal work, the conversations, the memory-making.

Your Scorecard: Rate 1-5:

- I am aware of the identity shift happening in me.
- I am finding small ways every day to be a partner, not just a caregiver.
- I have begun the legal work (POA, will, healthcare directive).
- I have asked for help from at least one outside person.

If your total is 12 or lower, the most important thing you can do this week is ask for help.

Key Terms to Know:

- Anticipatory grief: The grief that begins before the loss. Begins for the spouse at this stage, often unnamed.
- Identity shift: The slow transformation from spouse to caregiver. The single biggest threat to the marriage.
- Memory book: A collection of photos, letters, and mementos made while the person with dementia can still participate.

Reflection Questions:

1. What is the small ritual you can do today that keeps you a partner, not just a caregiver?

2. What legal work have you been putting off?
What is one document you can complete this week?

3. Who is the one person you can ask for help this week?

Action for This Week:

- Pick one legal document you have been delaying. Power of attorney, healthcare directive, will, or a list of accounts and passwords. Complete it this week, while your spouse can still sign.
- Pick one small ritual. A daily hand-hold. A weekly phone call to a grandchild, together. A Saturday morning breakfast. Do it every day this week.
- Ask one person for one specific thing. Could you sit with [spouse's name] for two hours on Tuesday so I can go to the doctor? Asking is not weakness. It is the recalibration in action.

Chapter 5: Husband, Wife, or Caregiver?

The question that no one asks out loud, and that every spouse in this position eventually has to answer, is this: Am I still a husband? A wife? A partner? Or am I now a caregiver? The answer is not either/or. The answer is both. But the balance between the two — the proportion of your day that is spent in the role of spouse versus the role of caregiver — is shifting. And the shift is one of the silent shapers of how the marriage survives this.

The role of spouse is the role you have been in for decades. It is the role of equal. The role of partner. The role of we. You make decisions together. You carry the load together. You argue, you make up, you negotiate, you lean on each other. The spouse role is a horizontal relationship — two adults side by side, each carrying their own weight and sharing the load.

The role of caregiver is a different relationship. It is vertical. The caregiver stands above the one being cared for. The caregiver makes decisions for, not with. The caregiver manages, monitors, administers, advocates. The caregiver carries the load. The caregiver's expertise grows, day by day, in things they never asked to be expert in — medications, side effects, behavioral triggers, social services, the medical system.

When a spouse becomes a caregiver, the marriage changes. The two-equal structure becomes a one-equal-and-one-helper structure. The horizontal becomes vertical. The partnership becomes, in some measure, a clinical relationship.

This change does not have to destroy the marriage. But it will reshape the marriage. And the reshaping is something you can — and must — be intentional about.

The trap: becoming only a caregiver.

The biggest risk in this transition is the slow disappearance of the spouse role. The caregiver role is so demanding, so absorbing, so constant, that it can quietly crowd out everything else. You become so focused on what they need — their medications, their meals, their appointments, their safety — that you forget that they also need you. The person who is their partner. The person who is not their nurse. The person who is still their husband or wife.

The signs that this trap is closing in:

- You talk to them mainly to give instructions. Take your pills. Eat your lunch. Let's go to the doctor.
- You have stopped asking their opinion on things. What do you want for dinner? has become We're having pasta.
- You have stopped sharing the small intimacies. The morning coffee together, with the newspaper. The evening TV show, side by side. The shared glance across the room.
- You have stopped touching them in non-caregiving ways. The hand on the shoulder as you pass. The kiss on the forehead. The hold in bed at night.
- You have started to think of them as a problem to manage, not a person to be with.
- You have started to refer to them, even in your own head, as the patient.

If any of these are familiar, the trap is closing. Not because you are a bad spouse. Because the disease is so demanding that the caregiving role can swallow everything if you let it.

The other trap: refusing to be a caregiver at all.

There is another trap, at the other end. The trap of refusing to take on the caregiver role at all. The trap of saying, I am their spouse, not their nurse. I will not do this. I will not become their parent. I will not lose myself in the role of caregiver.

This trap is more common than you think, and it is not just a failure of love. It is a failure to recognize that the marriage has changed, and that the new marriage requires new roles. The spouse who refuses to be a caregiver often ends up being neither spouse nor caregiver — they end up being a distant bystander, calling the doctor occasionally, showing up for the major decisions, but otherwise disengaged.

Both traps are dangerous. The first one destroys the marriage by removing the partnership. The second one destroys the marriage by removing the presence.

The middle way: holding both roles.

The middle way is to hold both. To be the spouse and the caregiver. To manage the medications and the morning coffee. To schedule the doctor and the weekly date night. To monitor the safety and the small intimacies.

This is not easy. It is, in fact, the most demanding emotional work you will ever do. It requires you to be aware of which role you are in, moment by moment, and to be intentional about preserving the spouse role even when the caregiver role is overwhelming.

The practical work of holding both:

1. Schedule the spouse role.

If you do not schedule it, it will not happen. The caregiving role is reactive — it responds to emergencies, to medications, to the next thing that needs to be done. The spouse role is proactive — it needs to be created, protected, scheduled.

What goes on the spouse schedule? The morning coffee, if that was a thing. The evening walk, if that was a thing. The weekly date night, even if it is just a takeout dinner and a movie at home. The shared glance. The shared joke. The I love you before bed, even if they cannot say it back.

Schedule these the way you schedule the medications. Treat them with the same seriousness. If you let them go, they will not come back.

2. Touch them, not just to care for them.

Caregiving touch is functional. The arm you take to help them stand. The shoulder you steady as they walk. The hand you hold to cross the street. These are necessary, and they are good. But they are not the only touch.

There is also the touch that is not functional. The hand on the cheek. The arm around the shoulder. The kiss on the forehead. The hand on the back of the neck. The lying next to each other in bed, simply present. This touch is the touch of the spouse, not the caregiver. It says: I am here, and I am not here because you need me. I am here because I want to be.

3. Ask their opinion, even when you know the answer.

The caregiving role is a role of expertise. You know what they should eat, when they should sleep, what they should wear. You know because you have been paying attention. The spouse role is a role of equality.

You ask their opinion, not because you need the answer, but because asking says: You are still my partner. Your preferences still matter. You are still in this marriage as a full person.

Ask them what they want for dinner. Ask them which grandchild they want to call. Ask them what music they want to listen to. Ask them where they want to sit. Ask them, in small things, again and again, even when you know the answer.

4. Tell them the things you have been meaning to tell them.

This is the work of the spouse role that only you can do. Not the medical work. Not the legal work. The relational work. The work of saying, before the slope takes the words away: I have been thinking about the time we went to [place]. It was one of the best days of my life. Thank you for that. Tell them the things you have been meaning to tell them. The thank-yous. The apologies. The small confessions. The love.

Tell them now. While they can still hear you.

5. Take breaks. From the caregiver role.

You cannot be the caregiver twenty-four hours a day. You will burn out. You will become bitter. You will start to resent the marriage. The only way to hold both roles for the long haul is to take breaks from the caregiving role, so that the spouse role can breathe.

The breaks can be small. An hour at the gym. A coffee with a friend. A walk alone. A visit to the children, alone. A weekend away, with a sitter or in a respite care facility. Whatever it takes to come back to the caregiving role with something left in you.

The breaks are not selfishness. They are the recalibration. They are what allow you to be both

spouse and caregiver, instead of only the second.

The marriage that survives this is the marriage in which both roles are held, intentionally, in tension. The marriage that does not survive is the marriage in which one role — usually the caregiver — takes over and the other disappears.

Hold both. The marriage depends on it.

Chapter 5 at a Glance

Key takeaway: You are both spouse and caregiver. The proportion is shifting, but both roles must be held. The marriage that survives holds both, intentionally, in tension.

5 Things to Remember:

1. The trap on one side: becoming only a caregiver, losing the partnership.
2. The trap on the other side: refusing to be a caregiver at all, becoming a distant bystander.
3. The middle way is to hold both, with awareness, moment by moment.
4. Schedule the spouse role. Treat it with the same seriousness as the medications.
5. Touch them for love, not just for caregiving. Ask their opinion. Tell them the things you have been meaning to say.

Your Scorecard: Rate 1-5:

- I am aware of which role I am in, moment by moment.
- I have scheduled at least one spouse-role activity this week.
- I am touching them in non-caregiving ways every day.

- I have asked for at least one break from caregiving in the past month.

If your total is 12 or lower, the most important thing is to schedule one spouse-role activity this week.

Key Terms to Know:

- Respite care: Short-term care provided to a person with dementia, giving the primary caregiver a break. Can be in-home, adult day care, or a short stay in a facility.
- Horizontal vs. vertical relationship: A marriage is horizontal (two equals). A caregiving relationship is vertical (one above the other). The challenge is to keep the marriage horizontal even as the caregiving becomes vertical.

Reflection Questions:

1. What is the small ritual that has fallen away from your marriage in the past six months?
2. What would it look like to schedule that ritual back in, the way you schedule medications?
3. When was the last time you touched your spouse in a non-caregiving way?

Action for This Week:

- Schedule one spouse-role activity. Write it on the calendar. Treat it as immovable.
- Touch your spouse in a non-caregiving way, every day this week. The forehead. The hand. The shoulder.
- Identify one person who can give you a break — a relative, a friend, a paid caregiver. Make the call. Ask for two hours. Use them for yourself.

Chapter 6: Telling the Children, the Friends, the World

The diagnosis is a private thing. It is yours and your spouse's. But the diagnosis is also a public thing. It will eventually need to be shared — with the children, with the friends, with the employer, with the world. The sharing is its own work, and it has its own costs.

Why the sharing is hard.

The diagnosis is a label. Once the label is out, it cannot be taken back. The people you tell will see your spouse differently. They will look for the symptoms. They will second-guess your spouse's words. They will treat them as less than they were. Some will be kind about it. Some will not.

You may want to protect your spouse from this. You may want to keep the diagnosis quiet, so that your spouse can continue to be seen as competent, as the person they were, as the spouse the world knows. This is a loving instinct. It is also, eventually, a losing one. The slope is going to be visible. The symptoms are going to be undeniable. The longer you wait to tell, the harder it becomes, and the more it looks like you were hiding something.

The diagnosis is also a thing you cannot keep private forever. The slope will require help. The slope will require the children to step in, the friends to understand, the neighbors to be patient, the medical system to engage. You cannot do all of this alone. And you cannot do all of this while keeping the diagnosis secret.

So the question is not whether to tell, but who, when, and how.

The children.

This is the hardest telling. The children are your children, but they are also adults with their own lives, their own fears, their own relationships with the parent who is now diagnosed. The diagnosis will land differently on each of them.

Some children will step up immediately. They will come to the next doctor's appointment. They will offer to take shifts. They will call every week to check in. These are the children who will be your allies in the long road ahead.

Some children will go into denial. They will say, Dad is just getting older. Don't make it a big deal. He's fine. These are the children who will be harder to engage, but who may come around when the slope becomes undeniable.

Some children will become angry. Why didn't you tell us sooner? Why are you making the decisions alone? What gives you the right? These are the children who are grieving, and who are expressing the grief as anger.

Some children will disappear. They will not be able to handle it. They will not return your calls. They will not visit. They will feel guilty about this, and the guilt will keep them away. Do not chase them. They will come back, or they will not. Either way, you cannot make this journey for them.

How to tell the children.

The conversation should be face to face if possible. A video call if distance prevents. A phone call if a video call is not possible. Not an email, not a text, not a

letter. They deserve the gravity of your presence.

The conversation should include:

- The diagnosis, in plain language. Mom has been diagnosed with Alzheimer's disease. It is a form of dementia. It is progressive. There is no cure.
- What you have seen. The patterns, the symptoms, the doctor's findings. Be specific, so they understand this is not a guess.
- What the plan is. The doctor's appointments, the medications, the legal documents, the home care. Show them you are not panicking. You have a plan.
- What you need. Be specific. I need you to come for a weekend once a month so I can rest. I need you to manage Mom's finances if I get sick. I need you to be in the room with me when we tell her doctor about the next stage.
- What you are asking of them. I am not asking you to be here every day. I am asking you to be in this with me, in the ways you can.

Do not ask them to be the spouse. Do not ask them to be the primary caregiver. They have their own lives. You are asking them to be the family.

The friends.

The friends are different from the children. The friends do not have the same obligation. The friends can be told selectively, and the timing is less urgent.

Tell the close friends first. The ones who have known you and your spouse for years. The ones who have seen the slope. They may already be wondering. They will be relieved to know.

Tell the casual friends later, when the slope makes it necessary. Until then, you are not obligated to disclose.

The employer.

If your spouse is still working, the diagnosis is going to affect their job. They may start to make mistakes. They may forget meetings. They may struggle with tasks that used to be easy. The employer will notice. The employer will, eventually, have to be told.

This is a difficult conversation. The Americans with Disabilities Act (ADA) and similar laws in other countries protect people with dementia from discrimination. Your spouse does not have to disclose unless they want to. But if they are starting to struggle, a conversation with HR, framed as a request for accommodation, may be wise.

If your spouse is no longer working, this conversation is unnecessary.

The world.

The world does not need to know, in detail. The world needs to know that your spouse has a medical condition, that you are the caregiver, and that you may need patience and accommodation. The world does not need to know the diagnosis, the medications, the prognosis.

The world needs to know: Please be patient. Please be kind. Please understand if I cancel. Please understand if I do not return your call. Please understand if I am not the person I used to be.

The cost of the telling.

Each telling is a small grief. You are, in each conversation, confirming that the diagnosis is real. You are, in each conversation, becoming the spouse of a person with a terminal illness. You are, in each conversation, losing the marriage that was, in the eyes of the people you are telling.

But the telling is also a relief. The loneliness of the diagnosis is, in part, the loneliness of secrecy. Once you start telling, the load is shared. The diagnosis becomes real, but the realness is a kind of permission. Permission to be tired. Permission to ask for help. Permission to be the spouse of someone with dementia, in the eyes of the people around you.

The telling is hard. The telling is necessary. The telling is the beginning of the public marriage — the marriage that exists not just between you and your spouse, but between you and your family, your friends, your community, your world.

You do not have to tell everyone. You do not have to tell everyone at once. But you have to tell the people who need to know. And the sooner you start, the less you carry alone.

Chapter 6 at a Glance

Key takeaway: The diagnosis will eventually need to be shared. The telling is hard, the cost is real, but the alternative — the loneliness of secrecy — is worse.

5 Things to Remember:

1. Tell the children face to face, with the diagnosis, the plan, and the specific help you need.
2. Tell the close friends selectively and as needed. The world does not need to know in detail.
3. The employer conversation is delicate but may be necessary. ADA and similar laws protect your spouse.
4. Each telling is a small grief, and a relief. The load becomes shared.
5. The public marriage is the marriage that includes the family, the friends, the community. It

is part of the new shape of the marriage.

Your Scorecard: Rate 1-5:

- I have told at least one of my adult children.
- I have identified the close friends who need to know.
- I have a plan for the employer conversation (if relevant).
- I have a clear sense of who does not need to know.

If your total is 12 or lower, the most important thing is to schedule the children conversation this week.

Key Terms to Know:

- ADA (Americans with Disabilities Act): US law that protects people with disabilities, including dementia, from discrimination. Requires reasonable accommodation in the workplace.
- HR (Human Resources): The department in a company that handles employee issues, including accommodations. The right place for the employer conversation.

Reflection Questions:

1. Which of your children is most likely to step up? Most likely to need time? Have you planned for both?
2. Who are the close friends who need to know this week?
3. What is the one thing you are most afraid of, in telling the world?

Action for This Week:

- If you have not yet told an adult child, schedule the conversation. Face to face or video call. Have the plan ready. Have the specific help you need

ready.

- Identify the one close friend you can tell this week. Have the conversation.
- If your spouse is still working and the slope is visible, research the ADA accommodation process. Just the research. The conversation with HR can come later.

Chapter 7: The Marriage That Was, and the Marriage That Is

There is a particular grief that belongs to the spouse of someone with dementia. It is the grief of watching the person you married become someone else, slowly, while they are still alive. It is the grief of losing the marriage that was, while you are still in the marriage that is. It is the grief that has no name, because the death has not yet come.

This chapter is about that grief, and about the work of mourning a marriage that is still happening.

The marriage that was.

Every couple has a marriage that was. The marriage of the early years. The marriage of the children. The marriage of the middle. The marriage of the last decade. The marriage of the trips, the holidays, the small rituals, the inside jokes, the shared history. The marriage that was is the marriage you remember. The marriage that was is the marriage that lives in the photographs, in the stories, in the shared language that only the two of you speak.

When dementia enters, the marriage that was does not disappear. It lives on, in you. It lives on in your memory, in your body, in the way you know how they take their coffee, in the way you know which story they will tell next. The marriage that was is still your marriage, in the sense that it shaped you, that it is part of you, that you are the person you are because of it.

But the marriage that was is no longer the marriage that is. The marriage that is has a different shape. The marriage that is has new roles, new dynamics, new losses. The marriage that is has a slope. The marriage that is has a future that is shorter, and stranger, and more uncertain than the future you had planned.

The grief is the work of holding both marriages at once. The marriage that was, which is the marriage of your memory. The marriage that is, which is the marriage of your daily life. The two marriages are both real. The two marriages are both yours. The two marriages are not the same.

The marriage that is.

The marriage that is has its own gifts, though they are hard to see at first. The marriage that is has a kind of intimacy that the marriage that was did not have. It is the intimacy of caretaking. It is the intimacy of being needed. It is the intimacy of watching someone you love in their most vulnerable state, and choosing to be there.

The marriage that is also has a kind of presence that the marriage that was did not have. The marriage that was was a partnership of two competent adults, each of whom had a life outside the marriage. The marriage that is is a partnership in which one of you is the entire world for the other. The marriage that is has a concentration, a focus, a single-mindedness that the marriage that was did not require.

The marriage that is also has a kind of grace. The grace of small moments. The hand that finds yours in the night. The smile that still arrives, even when the words do not. The laugh that still comes, even when the joke has been told a hundred times. The look in their eyes when you say their name, the one moment

of recognition that breaks your heart and heals it at the same time.

The marriage that is not better than the marriage that was. It is not worse. It is different. And the work of the spouse who is grieving is to honor both — to honor the marriage that was by remembering it, by telling the stories, by keeping the photographs — and to honor the marriage that is by being present in it, by finding the grace in it, by loving the person who is here now, not the person who used to be here.

The grief that has no name.

The grief of the spouse of someone with dementia is unusual because the person is still alive. There is no funeral. There is no sympathy card. There is no casserole from the neighbors. There is no permission to grieve, because the person is still here.

But the grief is real. It is the grief of losing the person you married, day by day, while they are still in the room. It is the grief of watching their personality change, their memories fade, their interests disappear. It is the grief of becoming a stranger to the person you knew best in the world.

This grief has a name in the literature: anticipatory grief. The grief that begins before the loss. The grief that is, in some ways, more difficult than the grief that comes after a death, because the death at least has a finality, a moment, a place to put the loss. Anticipatory grief is the grief that lives in the in-between, in the long slow goodbye that does not end.

How to mourn a marriage that is still happening.

The first thing to know is that the mourning is necessary. You cannot skip the grief and move to acceptance. The grief is the work, and the work takes

time. Years, sometimes. The grief does not get smaller. It gets walked through, day by day, until you come out the other side of it.

The second thing to know is that the mourning is not betrayal. You are not betraying your spouse by grieving the marriage that was. You are not betraying the marriage that is by remembering the marriage that was. The two are both real. The two are both yours.

The third thing to know is that the mourning has a shape. There is a pattern that most spouses in this position move through. The pattern is not linear. You will move back and forth. You will have bad days and better days. You will have weeks of acceptance and days of rage. But the pattern is generally this:

Denial. The slope is not real. The diagnosis is wrong. The doctor is being alarmist. The forgetting is just aging. The marriage is fine.

Anger. At the disease. At the doctors. At the unfairness. At the spouse who is leaving you without leaving. At the world that moves on while you are stuck. At the future that was taken.

Bargaining. If I just do everything right. If I just find the right doctor. If I just give them the right supplements. If I just love them enough. Maybe the slope will stop. Maybe the diagnosis will be reversed. Maybe the marriage will be what it was.

Depression. The weight of the slope. The tiredness that sleep does not fix. The loneliness that no one can see. The grief that has no name.

Acceptance. Not acceptance that the disease is good. Not acceptance that the marriage is what it was. Acceptance that the marriage is what it is. Acceptance

that the slope is real. Acceptance that you are the spouse who remembers, and that the remembering is the work of the rest of your life.

You will move through these stages, and you will move back. You will have days of acceptance and days of denial. You will have weeks of energy and days of collapse. The shape is not a line. The shape is a spiral, and you will return to the same places again and again, but each time at a slightly different level.

The work of remembering.

The marriage that was is worth remembering. The stories are worth telling. The photographs are worth keeping. The places you went are worth going back to, in memory. The songs you shared are worth playing. The inside jokes are worth laughing at, even if your spouse no longer understands the joke.

The remembering is not a betrayal. The remembering is the work of keeping the marriage that was alive, in the place where it can live — in you, in the family, in the shared history. The remembering is the work of saying: this person was here. This person was loved. This marriage was real. And it is still real, in me, even as the slope takes the words and the memories from them.

The work of being present.

The marriage that is worth being present in. The small moments. The hand on the hand. The look in the eye. The shared silence. The slow walk in the garden. The cup of tea, made the way they always liked it, even if they no longer know they liked it that way.

The being present is the work of saying: I am here. I am with you. I see you. You are still my spouse, and I am still your spouse, and we are still in this marriage,

in whatever form it now takes.

The two works — the remembering and the being present — are not in conflict. They are the same work, seen from two angles. The remembering honors the marriage that was. The being present honors the marriage that is. Together, they honor the whole of the marriage — the whole arc, from the first kiss to the last hand-hold, from the vows to the goodbye.

This is the work of the spouse who remembers. It is the work of a lifetime. It is the work that no one can do for you. It is the work that, in the end, is the marriage itself.

Chapter 7 at a Glance

Key takeaway: The grief of the spouse of someone with dementia is the grief of the marriage that was, while you are still in the marriage that is. Both marriages are real. Both deserve to be honored.

5 Things to Remember:

1. The marriage that was lives on in you. The marriage that is has its own gifts.
2. The grief has a name — anticipatory grief — and it is real.
3. The mourning is necessary, and it has a shape: denial, anger, bargaining, depression, acceptance.
4. The work of remembering honors the marriage that was.
5. The work of being present honors the marriage that is. Both are needed.

Your Scorecard: Rate 1-5:

- I have begun to name the grief I am feeling.

- I have found at least one way to remember the marriage that was.
- I have found at least one way to be present in the marriage that is.
- I have not been told to "move on" or "be strong" by anyone who does not understand.

If your total is 12 or lower, the most important thing is to name the grief. Say it out loud to someone who will listen.

Key Terms to Know:

- Anticipatory grief: The grief that begins before the loss. The grief of the marriage that was, while the marriage that is still happens.
- The five stages: Denial, anger, bargaining, depression, acceptance. The pattern that most spouses in this position move through, often in spiral rather than line.

Reflection Questions:

1. What is the small thing you miss most about the marriage that was?
2. What is the small thing you are most grateful for in the marriage that is?
3. Who is the one person you can talk to about the grief?

Action for This Week:

- Tell one story from the marriage that was. To a friend, to a child, to yourself in a journal. Keep the marriage that was alive in the telling.
- Do one small thing this week that is for the marriage that is. A hand held. A song played. A slow walk.
- If the grief is heavy, find one place where you can say it out loud. A friend. A counselor. A

support group. The grief needs a witness. The
grief needs a place to land.

Chapter 8: The Money Conversation Nobody Wants

There is a conversation that almost no one in this position wants to have, and that almost everyone in this position needs to have, and that is the conversation about money.

The conversation about money is the conversation about the cost of dementia care. The conversation about long-term care insurance, or the lack of it. The conversation about assets, about the house, about the retirement accounts, about the children's inheritance, about Medicaid, about who pays for what when the savings run out.

This is a conversation that the spouse who is well often avoids, because it is terrifying. The cost of dementia care in the United States, for example, can run from \$50,000 to \$100,000 or more per year, depending on the level of care and the location. Most families are not prepared for this. Most families have not had the conversation. Most families discover, in the middle of the crisis, that the savings are not enough, the insurance is not enough, the government benefits are not enough, and the only options are to spend down, to place the spouse in a substandard facility, or to bankrupt the family.

This is not a conversation you want to have. This is a conversation you must have. And the time to have it is now, while the spouse with dementia can still participate, while there is still time to plan, while the assets are still intact.

The cost of care.

Dementia care comes in three rough tiers, and the cost of each tier varies dramatically by location, by the level of care needed, and by whether the spouse is still at home or in a facility.

Tier 1: Home care. Your spouse stays at home, with the spouse as the primary caregiver, supplemented by part-time in-home help. The cost is mostly opportunity cost — the lost wages, the lost retirement contributions, the lost freedom of the well spouse — plus the cost of any paid in-home help. A part-time home health aide, in 2024, runs roughly \$20-35 per hour, depending on location. Full-time care, if you can afford it, runs \$200,000+ per year.

Tier 2: Adult day care + home care. Your spouse spends the day at an adult day care program, which provides socialization, activities, and supervision, and returns home in the evening. The cost of adult day care runs roughly \$50-150 per day, depending on location. Combined with part-time home care, this can run \$30,000-80,000 per year.

Tier 3: Residential care. Your spouse lives in a facility — assisted living, memory care, or a nursing home. The cost of memory care in 2024 runs roughly \$5,000-10,000 per month, or \$60,000-120,000 per year, depending on location. Nursing home care, for those who need it, runs even more.

The median duration of dementia from diagnosis to death is 4-8 years, though it can be much longer. The lifetime cost of care, then, can easily run \$300,000 to \$1,000,000, even with careful planning.

The financial picture.

Most families are not prepared for this. The median retirement savings of Americans aged 65-74, in 2024, is roughly \$200,000. The cost of one year of memory

care can eat through a quarter of that. The cost of five years of memory care can eat through all of it, and then some.

The options for paying are limited:

- Personal savings. The first line of payment. The savings will be drawn down, sometimes to zero.
- Long-term care insurance. If your spouse has it. The earlier it was purchased, the better the coverage. Most policies have a daily or monthly benefit cap, an elimination period (you pay out of pocket for the first 30-90 days), and a maximum lifetime benefit. Read the policy.
- Medicare. Does not cover long-term custodial care. Covers short-term skilled nursing care after a hospital stay (up to 100 days), and some home health care under specific conditions. Not a solution for ongoing dementia care.
- Medicaid. Covers long-term care for those who qualify, but only after the spouse has spent down most of their assets. The rules vary by state, but generally the well spouse is allowed to keep a limited amount of assets (the "community spouse resource allowance") and the house (in most cases). Medicaid planning is its own profession; consult an elder law attorney.
- Veterans benefits. If your spouse served in the military, the VA has programs that can help with long-term care. Aid and Attendance is the most common.
- Reverse mortgage. Allows you to tap the equity in your home without monthly payments, but the loan comes due when you move out, sell, or die. Can be a useful tool, but has costs and risks.

- Family contributions. The children may be able to contribute, but this is a sensitive area. Some families are able to do this. Some are not.

The conversation you must have.

The conversation you must have is the conversation with a financial planner who specializes in elder care, and an elder law attorney, ideally before you need them. The cost of the initial consultation is small. The cost of not having the conversation is enormous.

The conversation with the financial planner. Bring the diagnosis. Bring the assets. Bring the insurance policies. Ask: Given my spouse's diagnosis, what is the best use of these resources? What is the realistic cost of care over the next 5-10 years? What options do we have that we may not be aware of?

The conversation with the elder law attorney. Bring the diagnosis. Bring the family situation. Ask: What legal documents do we need? How do we qualify for Medicaid without bankrupting the well spouse? How do we protect the house? How do we structure the assets so that the well spouse is not left with nothing?

The conversation with your spouse.

This is the hardest conversation. Your spouse may be aware of the financial picture, or they may not. They may be able to participate, or they may not. If they can, the conversation should include them, even if only in part. The conversation might go like this:

We have been talking to a financial planner about how to handle the long road ahead. There are some decisions we need to make together, while you can still make them. I want to walk you through what we have learned, and I want your input on the big decisions.

Then walk them through it. The cost of care. The options. The decisions. Ask for their input. Honor their preferences. Make the decisions together, while you can.

If your spouse cannot participate, the conversation becomes your conversation. With the planners. With the attorneys. With the children. The decisions are still yours to make, but they are made alone, and they are made with the weight of knowing that the spouse is no longer able to be a partner in the decision.

The conversations that are easier to avoid.

There are other financial conversations that are easier to avoid, and that are also necessary:

- The conversation with the adult children about the cost of care, and about whether they can contribute. This is a hard conversation, but the children need to know what is coming.
- The conversation about the long-term care insurance policy, if there is one. Read the policy. Understand the elimination period. Understand the daily benefit. Understand the trigger for benefits (typically, the inability to perform two or more activities of daily living).
- The conversation about the will, the trust, the power of attorney, the healthcare directive. These documents need to be in place while the spouse can still sign them. This is the work of the first year, not the last.

The conversation you can keep having.

The money conversation is not a one-time event. It is an ongoing conversation. As the slope progresses, the financial picture changes. The savings go down. The cost of care goes up. The options narrow. The conversation has to be had again, and again, and

again.

You do not have to have the conversation alone. The financial planner, the elder law attorney, the geriatric care manager, the social worker at the hospital — these are the people who can help you have the conversation, and who can help you make the decisions that follow.

The money conversation is the conversation that no one wants. But the money conversation is the conversation that allows the marriage to survive. Because the marriage cannot survive if the savings are gone, the house is gone, and the well spouse is left with nothing. The marriage has to be protected, financially, so that the marriage can be protected, in every other way.

Have the conversation. Have it now. While there is still time to plan.

Chapter 8 at a Glance

Key takeaway: Dementia is expensive. Most families are not prepared. The conversation about money is hard, but it is the conversation that allows the marriage to survive.

5 Things to Remember:

1. The lifetime cost of dementia care can run \$300,000-\$1,000,000+.
2. Most families cannot afford this from savings alone. Long-term care insurance, Medicaid planning, VA benefits, and reverse mortgages are common tools.
3. The conversations you must have: with a financial planner, an elder law attorney, your spouse (if possible), and your adult children.

4. The legal documents — will, trust, POA, healthcare directive — must be done while the spouse can still sign them.

5. The money conversation is ongoing, not one-time. Have it again and again as the slope progresses.

Your Scorecard: Rate 1-5:

- I have consulted a financial planner who specializes in elder care.
- I have consulted an elder law attorney.
- I have read the long-term care insurance policy (if there is one).
- I have had the conversation with at least one adult child about the cost of care.

If your total is 12 or lower, the most important thing is to make one call this week — to a financial planner, an elder law attorney, or your Area Agency on Aging. Just the call.

Key Terms to Know:

- Long-term care insurance: Insurance that pays for long-term care, including memory care and nursing home care. Best purchased before diagnosis.
- Medicaid planning: The legal and financial work of structuring assets so that the spouse qualifies for Medicaid without being left destitute. Requires an elder law attorney.
- Aid and Attendance: A VA benefit for veterans and their spouses who need help with daily activities. Can pay for memory care.
- Power of attorney (POA): A legal document that gives someone else the authority to make financial decisions on your behalf. Critical to set up while

the spouse has capacity.

- Healthcare directive: A legal document that specifies what medical treatments you do or do not want, and who can make medical decisions on your behalf. Also called a living will.

Reflection Questions:

1. Do you have long-term care insurance? Have you read the policy?
2. What is the realistic cost of care over the next 5-10 years, given your current assets?
3. What is the one financial conversation you have been avoiding?

Action for This Week:

- Make one call. To a financial planner who specializes in elder care. To an elder law attorney. To your Area Agency on Aging (in the US) or its equivalent in your country. Just the call. Just the conversation. The action is to start the conversation, not to finish it.
- If you have long-term care insurance, pull out the policy. Read the daily benefit, the elimination period, the maximum lifetime benefit. Understand what you have.
- Make a list of the legal documents you need to put in place: will, trust, POA, healthcare directive. Check which ones you have. Schedule the conversations for the ones you don't.

Chapter 9: When They Forget Your Name

The day comes. You knew it was coming. You have been preparing for it, in some quiet part of your mind, for months or years. You have been bracing for the moment when the forgetting goes past the point of compensation, past the point of social cover, past the point of the everyday small forgettings that are part of the slope.

The day comes when your spouse looks at you, and does not know who you are.

The day comes in different ways. For some, it comes suddenly — they look up from the newspaper, and they are confused, and they ask, Who are you? as if a stranger has wandered into the house. For others, it comes gradually — the look in the eye changes, the recognition flickers, the name comes a half-second slower each time, until the half-second becomes a second, and the second becomes a forever.

When the day comes, you will feel something that has no name. You will feel the marriage break, and not break, in the same moment. You will feel the person you love look at you with no recognition, and you will feel the love, still, in you, undimmed. You will feel the loss of the person you married, while the person is still in the room. You will feel the absurdity of the situation — you are sitting across the kitchen table from the person you have loved for forty years, and they do not know your name.

This is the chapter about that day. And the days that follow.

The first time.

The first time it happens, you will be alone with them. They will look at you with no recognition. They will ask who you are. Or they will not ask. They will simply look, and the looking will not be the looking of a spouse. The looking will be the looking of a stranger.

You will feel the floor tilt. You will feel the breath go out of you. You will feel the tears start, somewhere behind your eyes, and you will fight them. You will try to hold yourself together. You will try to be calm. You will try to remember that this is the disease, not the person. You will try to remember that they cannot help it. You will try to remember all the things you have read, all the things the doctors have said, all the things the support group has told you.

None of it will help. The first time is the first time, and the first time breaks something in you that does not fully heal.

What you do in that moment matters, both for them and for you.

What to do in the moment.

Do not take it personally. It is the disease, not the person. They are not choosing to forget you. They are not choosing to look at you as a stranger. The disease is taking the recognition away, and the taking is no more voluntary than the taking of any other capacity.

Do not correct them. Do not say, I am your husband. I am your wife. I am the person you have lived with for forty years. The correction will not land. They will not believe you. They will be frightened, or angry, or confused. The correction does not help them remember. It just reminds them that they should be able to remember, and they cannot.

Do not panic. They are still in the room. They are still breathing. They are still the person you love, even if they cannot, in this moment, recognize you. The panic in you will transfer to them. They will feel your fear, and they will respond with their own fear.

Do not leave the room. Your instinct will be to flee. To go somewhere private and cry. To call someone. To be away from this moment. Resist the instinct. They are in this moment, and they need you to be in it with them, even if they do not know you are their spouse. They need a calm presence. They need someone who is not afraid of them. They need you, even if they do not know it is you.

What to say.

There is no perfect script. There is no wording that will make this moment okay. But there are some things that are better than others.

Say something simple. It's okay. I'm here. You're safe. Not I am your husband, don't you remember me? Just I am here. You are safe. This is what they need to hear. They are confused. They are frightened. They need to know that they are not alone, and that they are not in danger.

Use their name. Not yours. Theirs. Mary, it's okay. I'm here. Calling them by name gives them an anchor. It says: I know who you are, even if you do not know who I am. It is a gift you can give them, in this moment, when the disease is taking so much away.

Be present. Hold their hand. Touch their arm. Sit next to them. The touch is not a correction. The touch is a grounding. It says: I am here. You are not alone. The touch is the language that dementia does not take away, until the very end. Use it.

What not to say.

Do not say, Don't you remember me? The question is a wound, both to them and to you. They cannot remember. The question does not help them remember. The question only reminds them that they should be able to, and they cannot.

Do not say, It's me, your husband. We've been married for forty years. The fact is true, but the fact is not what they need. They need presence, not facts.

Do not say, This is the worst day of my life. They may sense your grief, and they will absorb it as their own. They will feel that they have done something wrong, even though they have not.

What to do afterward.

After the moment — after they have calmed, after you have been present, after the recognition has returned (or not) — you will need to take care of yourself. The first time, the second time, the hundredth time. Each time is a small death, and each small death requires a small mourning.

Find a private place. Cry. Call someone. Write it down. Do whatever you do to release the grief. The grief is real. The grief is yours. The grief deserves a witness, even if the witness is only you.

Then, when you are ready, return to the marriage. Return to the spouse who is still in the room. Return to the holding, the touching, the being present. Return to the work of being the spouse who remembers, even when the spouse does not remember you.

The days that follow.

The first time is not the last. The forgetting will come again, and again, and again. Each time, you will feel the loss. Each time, you will go through the small

death. Each time, you will come back to the marriage, with the grief still in you, and the love still in you, and the work still to be done.

The days that follow are not all bad. There will be days of recognition, when they know your name, when they call you by the right word, when the looking in their eyes is the looking of a spouse. There will be days of laughter, when the disease is in the background, and the person is in the foreground, and you can sit together and watch the birds and be, for a moment, the couple you used to be.

There will be days of grief, too. Days when the forgetting is constant. Days when they are not in the room with you, even when they are in the room with you. Days when you are alone in the marriage, holding a stranger's hand, and the marriage is a one-way conversation, and the spouse you remember is gone, again, in the body of the person in front of you.

The days that follow are the long middle. The long middle is the long goodbye. The long goodbye is the work of the spouse who remembers. You will do this work, day by day, for as long as the disease gives you.

The gift in the day.

There is a gift in the day, though it does not feel like a gift at first. The gift is this: the day clarifies. The day strips away the small things — the petty arguments, the unresolved grievances, the should-haves and the if-onlys — and leaves you with the big thing. The love. The presence. The willingness to be here, even when the here is hard.

The day your spouse forgets your name is the day you find out what your love is made of. The love that survives the day is the love that was always there, under the small things. The love that survives the day

is the love that does not need recognition to exist. The love that survives the day is the love that is, in some essential way, unconditional.

This is the love that you have been building, all the years of your marriage. The day reveals it. The day does not create it. The day shows you what was always there.

Hold the day. Hold the grief. Hold the love. They are all part of the same thing. The day is the marriage, in this new form. The marriage that includes the forgetting, and the love that includes the grief, and the work that includes the holding.

You are not the spouse who has been forgotten. You are the spouse who is still here. The still-here-ness is the work. The still-here-ness is the love.

Chapter 9 at a Glance

Key takeaway: The day your spouse forgets your name will come. The day breaks something in you that does not fully heal. The day also reveals what your love is made of.

5 Things to Remember:

1. The day will come. Prepare for it without dreading it.
2. In the moment: be present, use their name, say simple things, do not correct them.
3. After the moment: take care of yourself. Cry. Call someone. The grief is real.
4. The days that follow are the long middle. There will be good days and grief days. Both are part of the work.
5. The day reveals the love that was always there. The love does not need recognition to exist.

Your Scorecard: Rate 1-5:

- I have thought about how I will respond when the day comes.
- I have a plan for who I will call or what I will do afterward.
- I am practicing the simple phrases I will use: It's okay. I'm here. You're safe.
- I have found someone I can talk to about the day when it comes.

If your total is 12 or lower, the most important thing is to find one person you can call the day it happens.

Key Terms to Know:

- **Anosognosia:** The medical term for the lack of awareness of one's own condition. Common in dementia. The person does not know that they are forgetting.
- **Recognition failure:** The specific moment when the person no longer recognizes a familiar face. Different from forgetting a name. Different from forgetting a memory.

Reflection Questions:

1. What is the one thing you most fear about the day?
2. Who is the one person you will call, the day it happens?
3. What is the simple phrase you will say, when the day comes?

Action for This Week:

- Tell one person — a friend, a sibling, a counselor — that the day is coming, and that you want them to be the person you call. Get their commitment.

- Practice the simple phrases, alone, in the mirror. It's okay. I'm here. You're safe. Make the words automatic, so that when the day comes, you do not have to think.

- Begin the memory work. The Memory Book. The voice recordings. The photo albums. While they can still participate.

Chapter 10: The Anger You Can't Say

There is an anger in the caregiver's heart that no one talks about. It is the anger at the person who is leaving you. The anger at the disease that is taking them. The anger at the unfairness. The anger at the future that was supposed to be yours and is no longer. The anger at the people who say, I could never do what you do, as if you had a choice. The anger at the person in the mirror who is also, sometimes, the person you are angry at.

This chapter is about that anger. Because the anger is real. Because the anger is human. Because the anger, unacknowledged, will eat the marriage from the inside.

The anger no one talks about.

When a spouse becomes the caregiver of someone with dementia, the world sees the caregiver as saintly. As patient. As loving. As a model of self-sacrifice. The world projects, onto the caregiver, the expectation of grace under impossible pressure.

The world is wrong.

The caregiver is not a saint. The caregiver is a person. The caregiver is a person who is exhausted, who is lonely, who is grieving, who is scared, who is losing the person they love, day by day, while the person is still in the room. The caregiver is a person who is, sometimes, angry.

The anger comes from many places. It comes from the disease, which is unjust. It comes from the medical system, which is frustrating. It comes from the adult children, who are not helping enough. It comes from

the friends, who have stopped calling. It comes from the spouse, who is leaving without leaving.

The anger at the spouse is the hardest anger. It is the anger that the caregiver feels most ashamed of. It is the anger that no one acknowledges. It is the anger that, if not addressed, will destroy the marriage.

The anger at the spouse is not because of anything they have done. It is because of what the disease is doing. The anger is misdirected. The anger belongs to the disease, to the unfairness, to the future. But the disease is not in the room, and the unfairness is too big to yell at, and the future is too abstract to blame. So the anger goes to the nearest available target, which is the spouse.

The anger at the spouse can look like:

- Resentment that they are not trying hard enough to remember. Why can't you just try?
- Frustration that they are not grateful for the care. After everything I do, you don't even say thank you.
- Irritation at the small things. The repetition. The questions. The lost items. The accidents.
- Rage at the moment when the recognition fails. The moment when they look at you and do not know you.
- Bitterness that the marriage has become this. That the retirement is not the retirement you planned. That the love story is not the love story you told.
- A wish, sometimes spoken, sometimes only thought, that the disease would take them faster. So the suffering — yours and theirs — would be over.

The wish.

The wish is the darkest part of the anger. The wish that the disease would take them faster. The wish that the end would come. The wish that the suffering would end.

This wish lives in almost every caregiver's heart, at some point in the long road. It is not a wish that your spouse would die. It is a wish that the suffering would end. It is a wish that you did not know you had, and that you feel ashamed of, and that you cannot say out loud.

The wish is not a betrayal. The wish is a human response to impossible pressure. The wish does not make you a bad spouse. The wish makes you a human being.

But the wish is dangerous if it is not acknowledged. The wish, unacknowledged, becomes guilt. The guilt becomes shame. The shame becomes withdrawal. The withdrawal becomes the slow disappearance of the spouse who is grieving, in the marriage that is being lost to two losses — the loss of the spouse with dementia, and the loss of the caregiver to the guilt.

Naming the anger.

The first step in working with the anger is to name it. To say, out loud, to someone who will not judge, I am angry. I am angry at the disease. I am angry at my spouse. I am angry at the unfairness. I am angry at the future I will not have. I am angry at the children. I am angry at the friends. I am angry at the world.

The naming is not the resolution. The naming is the beginning. The naming is what allows the anger to be seen, and to be held, and to be worked with, instead of being hidden, and being shamed, and being acted out

in the small cruelties of caregiving.

The naming requires a witness. A friend. A counselor. A support group. A journal. A person who can hear the anger without flinching, who can hold the anger without judgment, who can say, Of course you are angry. Of course. What else could you be?

The four doors of the anger.

The anger has four doors. The anger goes out through the doors it can find. The work of the caregiver is to make sure the anger goes out through the right doors.

Door 1: The work door. The anger goes into action. The anger becomes the fuel for the doctor's appointments, the legal documents, the home modifications, the care plans. The anger says: I will not let this disease destroy us without a fight. I will organize. I will plan. I will be the spouse who acts.

Door 2: The body door. The anger goes into the body. The anger becomes the exercise, the walk, the gym, the run. The anger says: I have a body. I will use it. I will not let the anger turn to illness.

Door 3: The voice door. The anger goes out through the voice. The anger becomes the cry, the scream (in the car, alone, with the windows up), the conversation with a friend, the conversation with a counselor, the conversation with God, if God is the kind of God you can scream at. The anger says: I will not be silent. I will not pretend. I will say what is true.

Door 4: The wrong door. The anger goes out through the wrong door. The anger becomes the sharp word to the spouse. The anger becomes the impatience with the adult child. The anger becomes the snapping at the friend. The anger becomes the cruelty that you will later regret.

The work is to keep the anger out of Door 4 as much as possible. The work is to direct the anger to Doors 1, 2, and 3. The work is to recognize the moments when the anger is about to go through Door 4, and to redirect it.

The moments of door-4 danger.

The moments of door-4 danger are the moments when:

- You are tired. The sleep has not come. The night was long. The morning is coming.
- You are hungry. The meal has been skipped. The blood sugar is low.
- You are alone. The respite has not come. The break has not been taken.
- You are triggered. They have asked the same question for the tenth time. They have had the accident. They have said the cruel thing.
- You are in pain. Your back hurts. Your head hurts. The chronic something is acting up.

In these moments, the anger is most likely to go through Door 4. In these moments, you have to recognize the danger, and you have to take yourself out of the room, if you can. Walk around the block. Call the friend. Take ten deep breaths. Put the anger through Door 3 (voice) or Door 1 (work) instead of Door 4 (the spouse).

The apology.

When the anger goes through Door 4 — and it will, sometimes, despite your best efforts — the work of repair is the apology. The apology to the spouse, who may or may not understand the apology. The apology to yourself, who is doing the best you can in an impossible situation. The apology is not a failure. The

apology is the work. The apology is the door back to the marriage.

I am sorry I snapped at you. I am tired. I am doing the best I can. I love you. I am sorry.

The apology is the work. The work is the marriage. The marriage is the holding of the anger and the love at the same time. The marriage is the holding of the anger and the grief at the same time. The marriage is the holding of the anger and the presence at the same time.

The anger will not go away. The anger is part of the long road. The anger is part of the work. The anger is part of being a human being, in an impossible situation, loving someone who is leaving.

The anger is the cost. The love is the work. Both are real. Both are honored.

Chapter 10 at a Glance

Key takeaway: The anger is real. The anger is human. The anger, unacknowledged, will eat the marriage. The work is to name the anger, to direct it through the right doors, and to apologize when it goes through the wrong one.

5 Things to Remember:

1. The anger is real. The anger is human. The anger is not a betrayal.
2. The wish that the suffering would end is the darkest part of the anger. It lives in almost every caregiver's heart.
3. The naming is the beginning. Find a witness. A friend, a counselor, a support group.
4. The four doors: work, body, voice, wrong. The work is to keep the anger out of Door 4.

5. The apology is the work. The marriage is the holding of the anger and the love at the same time.

Your Scorecard: Rate 1-5:

- I have named the anger, at least to myself, and ideally to a witness.
- I have identified my own door-4 danger moments.
- I have a plan for redirecting the anger in those moments.
- I have not yet taken the anger out on my spouse in a way I cannot repair.

If your total is 12 or lower, the most important thing is to find one person who can hear the anger. The anger needs a witness.

Key Terms to Know:

- Compassion fatigue: The exhaustion of caring, especially for someone with a progressive disease. The caregiver runs out of the emotional fuel for empathy.
- The four doors of the anger: Work, body, voice, wrong. A framework for directing the anger productively.
- The wish: The wish that the suffering would end. Lives in almost every caregiver's heart at some point. Not a betrayal. A human response.

Reflection Questions:

1. What is the small thing that triggers the anger most reliably?
2. Who is the one person you can tell the anger to?
3. What is the moment when the anger is most likely to go through Door 4?

Action for This Week:

- Find one person who can hear the anger. Tell them: I need to tell you something I have not been able to say. I am angry. I am angry at the disease. I am angry at my spouse. I am angry at the unfairness. I need you to just hear it.
- Identify your own door-4 danger moments. Write them down. Make a plan for those moments: walk, call, breathe, leave the room.
- Apologize for one thing you have been meaning to apologize for. To your spouse, to a child, to yourself. The apology is the work. The work is the marriage.

Chapter 11: The Family That Divides

There is a particular kind of suffering that belongs to the spouse of someone with dementia, and that is the suffering of the family that divides around the diagnosis. The family that was, before the diagnosis, a unit. The family that, after the diagnosis, becomes a collection of individuals, each with their own grief, their own fear, their own reaction, their own way of being in the relationship with the parent who is leaving.

This chapter is about that family. And about the work of holding the family together, or, when that is not possible, of holding yourself together while the family divides.

Why the family divides.

The diagnosis of a parent with dementia is a stressor that tests every relationship in the family. The siblings who were close become estranged. The siblings who were estranged become enemies. The adult children who lived nearby become exhausted. The adult children who lived far away become guilty. The in-laws become awkward. The grandchildren become confused. The family system that was, in any equilibrium, becomes a system in crisis.

The reasons are many:

The geography divide. One sibling lives nearby. The other lives across the country. The sibling who lives nearby becomes the primary caregiver, by default, even if no one ever explicitly assigned the role. The sibling who lives far away feels guilty, and the guilt expresses itself as criticism, or as over-involvement

from a distance, or as withdrawal. The nearby sibling resents the criticism and the lack of help. The far-away sibling resents the resentment. The family divides around geography.

The gender divide. In many families, the caregiving role still falls disproportionately to women — daughters, daughters-in-law, and wives. The sons may offer to help, but the help is often episodic, or financial, or managerial. The actual hands-on care — the bathing, the dressing, the feeding, the companionship — falls to the women. The women in the family feel overburdened. The men in the family feel accused. The family divides around gender.

The historical divide. The family has a history. The sibling who was the favorite, the sibling who was the difficult one, the sibling who married the wrong person, the sibling who was always closer to the parent who is now diagnosed. The history rises up in the crisis. The old wounds reopen. The old patterns reassert themselves. The family divides around the history.

The grief divide. Each person in the family is grieving the parent who is leaving. But each person is grieving in their own way, in their own time, in their own private hell. The sibling who was closest to the parent is grieving the most. The sibling who was most distant is grieving the least. The sibling who was the parent's favorite is grieving the loss of the favorite. The sibling who was not the favorite is grieving the loss of the chance to become the favorite. The family divides around the grief.

The control divide. The diagnosis brings decisions. Where will the parent live? Who will be the primary caregiver? When will the parent stop driving? When will the parent be placed in care? How will the

parent's money be spent? Each decision is a small power struggle. The family divides around the control.

The spouse's position.

You, as the spouse, are in a particular position. You are the one who is closest to the diagnosis. You are the one who is most affected. You are the one who has been doing the work of the long road. You are also, in many ways, the one who is least able to advocate for yourself.

You are exhausted. You are lonely. You are grieving. You are the one who is in the room with the spouse every day, and every night, and every moment in between. You are the one who is doing the work that no one sees, and that no one can fully understand, and that no one can fully thank you for.

You are also, in many ways, the one who is most vulnerable to the family dynamics. The siblings can argue with each other. The siblings can criticize each other. The siblings can gang up on each other. You are, in many cases, the one who is ganged up on. You are the one who is criticized. You are the one who is told you are doing it wrong, that you should do it differently, that you should put them in care, that you should not put them in care, that you should try this supplement, that you should try that doctor, that you should be more patient, that you should be less patient.

The criticism is almost always about the family member's own grief. The family member who criticizes is the family member who is most afraid. The criticism is the way the fear gets expressed.

The work of holding the family together.

You do not have to hold the family together. The family is not your responsibility. The family is a system of adults, and each adult is responsible for their own behavior. You are responsible for your own behavior, and for the care of your spouse. You are not responsible for the behavior of your adult children, or for the resolution of their old wounds, or for their grief.

That said, there are things you can do, if you have the energy, to keep the family from dividing too badly.

Hold the family meetings. When the major decisions need to be made, call a family meeting. In person if possible. By video call if not. The meeting should have an agenda, a time limit, and a clear decision to be made. The meeting should not be a place to process the old wounds. The meeting should be a place to make a decision, together, and to leave with a clear plan.

Assign the roles. In the meeting, ask each family member what they can do. Not what they should do. What they can do. Be specific. Can you take Mom on Saturday mornings so I can sleep in? Can you manage her finances? Can you be the one who calls the doctor when I am in the middle of something? Can you fly in once a quarter for a long weekend? The asking is not the answer. The answer is what each person says yes to.

Document the decisions. After the meeting, write down the decisions, and the assignments, and the timeline. Send the notes to each family member. The documentation prevents the I thought we agreed and the I never said I would do that. The documentation is the work of the marriage, extended to the family.

Hold the boundaries. When a family member is being critical, hold the boundary. I hear you. I know you have concerns. I am doing the best I can. I am not going to defend every decision. If you want to be helpful, here is what I need. If not, I understand, but I cannot take the criticism right now.

Let some things go. You do not have to win every argument. You do not have to be right about every decision. You do not have to convince every family member. You have to make the best decisions you can, with the information you have, and you have to let the family members who disagree, disagree. The energy you save by not fighting is energy you can spend on your spouse.

The work of letting go.

There will be family members who cannot be held. The sibling who is too critical. The sibling who is too absent. The sibling who is too angry. The family member who cannot be in this with you, in the way you need them to be.

You will have to let them go. Not forever. Not with anger. Just with the recognition that they are doing the best they can, and the best they can is not enough. You will have to grieve their absence, even before they have fully gone. You will have to make the marriage work without them, and you will have to keep the door open, for the day when they are ready to come back.

The family that divides is a real grief. The family that was, before the diagnosis, will not be the family that is, after. The family that is will be smaller, in some ways, and stronger, in other ways. The family that is will be the family that showed up. The family that is will be the family that did the work. The family that is will be the family that you can count on, in the end, to

be there.

The family that divides is a cost of the diagnosis. The family that survives is a gift.

Chapter 11 at a Glance

Key takeaway: The diagnosis divides the family. The divisions are real, and they are not your fault. The work is to hold the family together when you can, and to let go when you must.

5 Things to Remember:

1. The family divides around geography, gender, history, grief, and control. These are the predictable stressors.
2. The criticism from family members is almost always about their own fear, not about your performance.
3. The work of holding the family: family meetings, role assignments, documented decisions, held boundaries.
4. The work of letting go: there will be family members who cannot show up. Grieve their absence, keep the door open.
5. The family that survives is the family that showed up. The family that is will be smaller and stronger.

Your Scorecard: Rate 1-5:

- I have called at least one family meeting in the past six months.
- I have assigned specific roles to family members.
- I have held at least one boundary with a critical family member.

- I have begun to let go of the family members who cannot show up.

If your total is 12 or lower, the most important thing is to call one family meeting. Set an agenda. Make one decision. Document it.

Key Terms to Know:

- Family meeting: A structured conversation with the adult children and other key family members, focused on a specific decision or set of decisions. Should have an agenda, a time limit, and a clear outcome.
- Held boundary: A clear statement of what you will and will not accept, said calmly, without anger, without apology. I hear you. I am doing the best I can. Here is what I need.

Reflection Questions:

1. Which family member is most critical? Most absent? Most supportive?
2. What is the one decision the family needs to make together, that you have been avoiding?
3. What is the one family member you need to let go of, in order to keep the marriage?

Action for This Week:

- Schedule one family meeting. Set an agenda. Make one decision. Document it.
- Identify the most critical family member. Plan how you will hold the boundary. I hear you. I am doing the best I can.
- Identify the most absent family member. Plan how you will let go. Write a letter, in your journal, that you do not send. The letter is the work of grief.

Chapter 12: The Decision You Don't Want to Make

There is a decision that almost every spouse in this position eventually faces, and that almost no one wants to make, and that no one can make for you. It is the decision about whether to place your spouse in a care facility.

The decision is not whether to care for your spouse. That decision is made. You are caring. You have been caring. You will keep caring. The decision is about where the care happens. At home, with you as the primary caregiver, supplemented by in-home help. In an adult day care program, with evenings and weekends at home. In an assisted living facility, with a memory care unit. In a nursing home, for the spouse who needs skilled nursing.

The decision is also not a one-time decision. It is a series of decisions, made over the long middle, as the slope progresses. The decision to bring in part-time in-home help. The decision to enroll in adult day care. The decision to install a home security system, because of the wandering. The decision to stop them from driving. The decision to place them in a facility. The decision to move them from one facility to another, as the disease progresses.

Each decision is its own small death. Each decision is its own piece of the marriage that was, becoming the marriage that is.

This chapter is about how to make the decisions. How to know when the time has come. How to evaluate the options. How to make the decision without it destroying you.

The question of timing.

The most common question, when the diagnosis is still new, is: When is the right time to place them in care?

There is no single right time. The right time is different for every couple, for every family, for every financial situation, for every medical situation. The right time is the time when the cost of keeping them at home — to you, to them, to the marriage — exceeds the cost of placing them in a facility.

The cost is not just financial. The cost is:

- The cost to you. Your health. Your sleep. Your ability to function. Your own mental state. If you are breaking, the marriage is breaking.
- The cost to your spouse. Their safety. Their socialization. Their quality of life. A facility, even a good one, can sometimes provide more socialization, more activity, more stimulation than a single caregiver at home can.
- The cost to the marriage. The marriage is being lost, in the all-consuming nature of the caregiving. The marriage needs you to be something other than a caregiver, sometimes. The facility, paradoxically, can sometimes save the marriage, by taking the caregiving role off your shoulders, and giving you back the spouse role.

The signs that the time has come:

- You are no longer able to keep them safe. The wandering. The falls. The leaving the stove on. The leaving the house. The forgetting to eat, or eating things that are not food.
- You are no longer able to keep yourself healthy. The sleep deprivation. The weight loss. The depression. The medical problems that are

starting to show up.

- The medical needs have exceeded what you can provide at home. The injections. The wound care. The behavioral issues that require trained staff.
- They are no longer recognizing you, or the home, as familiar. The familiar places have stopped being the anchor. The familiar people have stopped being the anchor. They may, paradoxically, do better in a structured environment with trained staff.
- You have tried the in-home help, the adult day care, the part-time care, and the needs have continued to exceed what you can provide.

The grief of the decision.

The decision to place your spouse in a facility is a grief that has no name. It is the grief of letting go of the promise to keep them home. The grief of admitting that you cannot do it. The grief of seeing the marriage that was, become the marriage that is, in the form of a facility room and a daily visit.

The grief is real. The grief is yours. The grief deserves to be honored.

The grief is also, sometimes, the beginning of a different marriage. The marriage in which you are no longer the 24-hour caregiver. The marriage in which you visit, and hold their hand, and tell them the stories. The marriage in which the touch is the touch of a spouse, not the touch of a nurse. The marriage that has been saved, paradoxically, by the decision to let go of the caregiving.

The decision-making process.

The decision is not made alone. The decision is made with:

- The medical team. The neurologist, the geriatrician, the social worker. They can tell you what level of care is needed.
- The family. The adult children. The siblings. The friends. They can tell you what they can contribute.
- The financial planner. The elder law attorney. They can tell you what you can afford.
- The geriatric care manager. These professionals specialize in evaluating the situation, recommending options, and helping the family through the transition. The cost of an initial consultation is small. The value is enormous.

The visit.

Once the decision is made, the visit is its own work. The first visit, in particular, is hard. You are walking into a facility, with your spouse, to look at a room, or an apartment, or a memory care unit. The visit is, in some ways, a confirmation that the disease has won a battle. The visit is also, in other ways, a confirmation that you are doing the right thing.

What to look for in a visit:

- The staff. Are they kind? Are they patient? Do they look the residents in the eye? Do they call them by name? Do they have time to sit, or are they rushing from task to task?
- The residents. Are they clean? Are they dressed? Are they engaged in activities? Are they sitting alone, staring? Are they calling out, unattended?
- The environment. Is it clean? Is it bright? Is it calm, or is it chaotic? Are there places to walk, to sit, to be alone, to be with others? Is there a garden, a common area, a dining room?

- The food. Is it fresh? Is it varied? Are there options for the residents who have dietary restrictions?
- The activities. Is there a daily program? Are there activities that would appeal to your spouse? Music, art, gardening, pet therapy?
- The smell. Any good facility will have some smell, but it should not smell like urine, or rotting food, or chemical cleaning products.

The transition.

The transition, when it happens, is its own work. The first days are the hardest. Your spouse may be confused, frightened, angry. They may beg to come home. They may accuse you of putting them away. They may cry. They may shut down.

The transition is also a beginning. The beginning of a new arrangement, in which you are no longer the 24-hour caregiver. The beginning of a new marriage, in which the touch is the touch of a spouse, not the touch of a nurse. The beginning of a new life, in which you have time to sleep, to eat, to exercise, to be a person again, in addition to being a spouse.

The visit after the transition.

The first visit after the transition is its own small grief. You are walking in, as a visitor, to the facility where your spouse now lives. You are not the caregiver. You are the spouse. The visit is the marriage, in its new form. The visit is the work of being the spouse, in the way that the caregiving role no longer allows.

What to do on the visit:

- Be present. Not on the phone. Not in a hurry. Not thinking about the next appointment. Present. With them.

- Touch them. Hold their hand. Touch their face. Sit next to them. The touch is the language of the marriage, in any form.
- Talk to them. Tell them the stories. Tell them the news. Tell them about the children, the grandchildren, the friends. Even if they cannot understand, the words are the touch. The words are the presence.
- Bring the small things. The photo album. The favorite music. The familiar blanket. The familiar object from home. The small things are the anchors, in a place that is not home.
- Leave with grace. When the visit is over, say goodbye. I am going now. I will be back tomorrow/I will be back on Tuesday. I love you. The goodbye is the work. The goodbye is the holding, even when you are leaving.

The marriage after the decision.

The marriage after the decision to place your spouse in care is not over. The marriage has changed, again. The marriage is now, in some essential way, a long-distance marriage. You are in your home, or in your own space. They are in the facility. You visit. You call. You hold the hand. You tell the stories.

The marriage is also, in some ways, a new marriage. A marriage in which the caregiving is no longer your full-time work. A marriage in which the touch is the touch of a spouse, not the touch of a nurse. A marriage in which the small moments are the moments, and the long goodbye is the long goodbye, and the work is the work of being the spouse who remembers, in whatever form the marriage now takes.

The decision is hard. The decision is necessary. The decision is the work.

Chapter 12 at a Glance

Key takeaway: The decision about whether (and when) to place your spouse in care is the hardest decision of the long road. The decision is not whether to care. The decision is where the care happens. The decision is a series of decisions, made over time.

5 Things to Remember:

1. The right time is when the cost of keeping them home — to you, to them, to the marriage — exceeds the cost of placement.
2. The signs: you cannot keep them safe, you cannot keep yourself healthy, the medical needs exceed what you can provide, the home is no longer the anchor.
3. The grief is real. The decision is a grief that has no name.
4. The decision is not made alone. Medical team, family, financial planner, elder law attorney, geriatric care manager.
5. The marriage after the decision is not over. It is a new marriage, in which the touch is the touch of a spouse, not the touch of a nurse.

Your Scorecard: Rate 1-5:

- I have consulted a geriatric care manager about the decision.
- I have visited at least one facility (even if just to gather information).
- I have begun the conversation with my family about the decision.
- I have begun to make peace with the possibility that placement is the right choice.

If your total is 12 or lower, the most important thing is to schedule one visit to a facility, just to see what they look like. The visit is information. The information is power.

Key Terms to Know:

- Geriatric care manager: A professional who specializes in evaluating the situation, recommending options, and helping families through the transition to care.
- Memory care unit: A specialized unit within an assisted living facility or nursing home, designed specifically for people with dementia. Typically has higher staff-to-resident ratios, more security, and more structured activities.
- Respite care: Short-term care (days or weeks) provided to a person with dementia, giving the primary caregiver a break. Often a good first step before a permanent placement.

Reflection Questions:

1. What is the cost, to you, of continuing to be the 24-hour caregiver?
2. What is the cost, to your spouse, of the care you are able to provide at home?
3. What would it look like to make this decision, with the help of a geriatric care manager?

Action for This Week:

- Schedule a visit to one facility. Just one. Just to see.
- Talk to a geriatric care manager. The consultation is usually free or low-cost. The information is invaluable.
- Talk to one family member about the decision. Not to make the decision together, but to put the

decision on the table. I have been thinking about the long road. I want to talk to you about the possibility of placement. Not today. Not this month. But at some point. I want you to be in this conversation with me.

Chapter 13: Intimacy When They Don't Know Your Name

There is a chapter in the long road that no one writes about, and that almost every spouse in this position has to walk through, and that is the chapter about intimacy. The chapter about the body. The chapter about touch. The chapter about the marriage bed, and the marriage touch, and the marriage physical life, in the long slow goodbye of dementia.

This chapter is about that. Because the body does not forget as fast as the mind. Because the marriage, even when the mind is leaving, is still in the body. Because the touch is the language that dementia does not take away, until the very end.

The slow change.

The change in the physical marriage is not sudden. It is slow, the way the disease is slow. It begins with the small things. The hand that used to find yours in the night, finding yours less often. The kiss that used to be the morning greeting, becoming perfunctory. The touch on the shoulder as you pass in the kitchen, becoming less frequent. The lovemaking, becoming less frequent, then less frequent still, then stopping.

The change is not a rejection. The change is the disease. The change is the brain, no longer initiating the things the brain used to initiate. The change is the body, no longer recognizing the cues the body used to recognize. The change is the marriage, in the body, becoming something different from the marriage that was.

The change is also, for the well spouse, a small death. Each small death is a small grief. The hand that does not find yours. The kiss that is not given. The lovemaking that does not happen. Each is a small loss of the marriage that was, and a small step into the marriage that is.

The holding of hands.

The first thing to know is that the touch does not have to be the lovemaking. The touch can be the small things. The hand on the hand. The arm around the shoulder. The head on the chest. The lying next to each other in bed, simply present.

The small touch is the language of the marriage, in any form. The small touch is the language that dementia does not take away, until the very end. The small touch is the language of the spouse who remembers, to the spouse who is forgetting.

The small touch is also, for the well spouse, the work of being present. The small touch is the act of choosing to be in the marriage, in whatever form the marriage now takes. The small touch is the act of saying, with the body: I am here. You are not alone. We are still in this.

The change in the lovemaking.

The lovemaking will change. The change is not a failure of the marriage. The change is the disease.

The change may be that the spouse no longer initiates. The initiation was a brain act, and the brain is changing. The spouse may no longer recognize the cues. The cue was a memory, and the memory is changing. The spouse may no longer be able to participate. The participation required a coordination, and the coordination is changing.

The change may also be that the spouse becomes confused, or frightened, during the lovemaking. The confusion is the disease, expressing itself in a moment of intimacy. The spouse may not understand what is happening. The spouse may react with fear, or with anger, or with withdrawal.

The change is hard. The change is a grief. The change is the marriage that was, becoming the marriage that is.

What to do with the change.

The first thing is to grieve. The grieving is real. The grieving is yours. The grieving deserves a witness, and a place to land. The grieving is the work of the small death, and the small death is part of the long road.

The second thing is to communicate. With the spouse, if they can still communicate. I miss the lovemaking. I miss the touch. I want to be close to you, in whatever way we can be close now. The communication is not about the act. The communication is about the closeness. The closeness can take many forms, and the forms can change.

The third thing is to find the new forms. The new forms may be:

- The hand on the hand, in the night.
- The head on the chest, in the quiet.
- The bath together, where the bathing is the touch.
- The massage, where the touch is the caregiving.
- The kiss, when the kiss is welcome.
- The cuddling, when the cuddling is the closeness.

The new forms are not the lovemaking that was. The new forms are the lovemaking that is. The new forms

are the marriage, in the body, in the new shape.

The hand on the hand.

I want to say something specific about the hand on the hand. The hand on the hand is, in some ways, the most important touch. The hand on the hand is the touch that can happen at any time. The hand on the hand is the touch that does not require coordination, or memory, or initiation. The hand on the hand is the touch that is always available, and that always lands.

The hand on the hand is the touch of the marriage that was, in the marriage that is. The hand on the hand is the touch of the spouse who remembers, to the spouse who is forgetting. The hand on the hand is the touch of the love that does not need recognition to exist.

The bath together.

The bath together is, in some ways, a lost art. The bath together was once part of the marriage that was. The bath together is a way of being close, in a way that is intimate, but not necessarily sexual. The bath together is a way of being present, with the body, in a way that is caregiving, but not necessarily clinical.

The bath together is also, in many cases, a way of being close to the spouse who is in a facility. The bath together, in the facility, is the touch that the spouse may not be able to initiate, but that the spouse can receive. The bath together, in the facility, is the act of being present, in a way that the staff cannot be, because the staff are not the spouse.

The marriage that is, in the body.

The marriage that is, in the body, is not the marriage that was, in the body. The marriage that is, in the body, has a different shape. The marriage that is, in the body, is a marriage of small touches, of holding, of

being present. The marriage that is, in the body, is a marriage that does not require the spouse to be able to remember, or to initiate, or to participate. The marriage that is, in the body, is a marriage that is held by the well spouse, for both of them, in whatever form the body can still receive.

The marriage that is, in the body, is not less than the marriage that was. The marriage that is, in the body, is the marriage that survives. The marriage that is, in the body, is the love that does not need recognition to exist.

The grief of the body.

The grief of the body is real. The grief of the body is the grief of the lovemaking that was, and the lovemaking that is not. The grief of the body is the grief of the touch that was, and the touch that is now the touch of the caregiver, not the touch of the spouse. The grief of the body is the grief of the marriage bed, in the marriage that was, and the marriage bed that is now the marriage bed in the marriage that is, with the spouse who is no longer the spouse, in the body that the body still recognizes.

The grief of the body is also the grief of the well spouse's body. The body of the well spouse, who is still a sexual being, who still has the desires, who still has the longings, who still has the loneliness of the body, in the marriage that is. The body of the well spouse, who is grieving the lovemaking that was, and the touch that was, and the closeness that was.

The grief of the body is real. The grief of the body is yours. The grief of the body deserves to be honored.

The companion.

The well spouse, in the long road, may find that the grief of the body is more than they can hold alone. The well spouse may find that the loneliness of the body, in the marriage that is, is more than they can sustain. The well spouse may find that the companion — the friend, the lover, the partner in the body — is a need that the marriage that is, cannot meet.

The companion is a complicated decision. The companion is a decision that each spouse must make, in their own time, in their own way. The companion is not a betrayal, if the marriage that was has already been lost, in the body, in the way that the marriage that is has lost it. The companion is, sometimes, the way the well spouse survives, in the long road, in the marriage that is.

The companion is also, sometimes, a decision that the well spouse makes later, in the marriage that is, after the spouse has been placed in care, or after the spouse has died. The companion is, sometimes, the beginning of a new marriage, in a new form, after the long goodbye is over.

The companion is a decision. The companion is not a requirement. The companion is not a failure. The companion is a possibility, in the long road, for the spouse who is still here, and who is still a person, and who still has the needs of the body, in a marriage that is, in the body, no longer what it was.

The marriage in the body, at the end.

The marriage in the body, at the end, is the marriage of the touch that does not require the spouse to be able to respond. The marriage in the body, at the end, is the touch of the hand on the hand, in the night, in the facility, in the quiet. The marriage in the body, at the end, is the touch of the forehead on the forehead,

in the last hours, in the last breath.

The marriage in the body, at the end, is the marriage that has been held, by the well spouse, for both of them, in whatever form the body could still receive.

The marriage in the body, at the end, is the love that has survived the long road. The love that did not need recognition to exist. The love that did not need the lovemaking to be the marriage. The love that did not need the spouse to be the spouse, in the body, to be the marriage.

The love, in the end, is the marriage.

Chapter 13 at a Glance

Key takeaway: The body does not forget as fast as the mind. The marriage, in the body, survives in the small touches — the hand on the hand, the head on the chest, the bath together. The new forms of intimacy are the marriage, in this new shape.

5 Things to Remember:

1. The change in the physical marriage is the disease, not a rejection. Grieve the small deaths.
2. The small touches — the hand on the hand, the kiss, the cuddling — are the language of the marriage, in any form.
3. The lovemaking will change. The change is the grief. The new forms are the marriage that is, in the body.
4. The companion, for some spouses, is a need that the marriage that is cannot meet. The companion is a decision, not a betrayal.
5. The marriage in the body, at the end, is the love that has been held by the well spouse, for both of them, in whatever form the body could still

receive.

Your Scorecard: Rate 1-5:

- I am finding the new forms of touch in the marriage that is.
- I am touching my spouse in non-caregiving ways, every day.
- I have not yet had to grieve the loss of the lovemaking that was — or I have grieved, and I have begun to find the new forms.
- I have given myself permission to have the needs of the body, in the marriage that is.

If your total is 12 or lower, the most important thing is to touch your spouse today, in a non-caregiving way. The forehead. The hand. The shoulder. The touch is the language.

Key Terms to Know:

- Anticipatory grief: The grief that begins before the loss. The grief of the body, in the marriage that is, is a form of anticipatory grief.
- The hand on the hand: The most important touch. Available at any time. Does not require memory, or coordination, or initiation. The touch of the marriage that is, in the body.
- The companion: A friend, a lover, a partner in the body, for the well spouse. A complicated decision. Not a betrayal, if the marriage that is, in the body, has already been lost. Not a requirement. A possibility.

Reflection Questions:

1. What is the small touch that you have stopped giving, in the long road?
2. What is the new form of closeness that you have found, in the marriage that is?

3. What is the grief of the body, that you have not yet honored?

Action for This Week:

- Touch your spouse today, in a non-caregiving way. The forehead. The hand. The shoulder. The touch is the language.
- Have the conversation, with yourself, about the grief of the body. Name it. Honor it. Let it land.
- If the loneliness of the body is becoming unsustainable, find one person you can talk to about it. A friend, a counselor, a support group. The loneliness needs a witness.

Chapter 14: The Stranger in Your Bed

There is a phase in the long road that no one names, and that almost every spouse eventually walks through, and that is the phase of living with a stranger.

The stranger is the person who looks like your spouse. The stranger has the same body, the same face, the same hands that held yours for forty years. The stranger is in your bed, or in your home, or in the facility room you visit. The stranger is the person you married, and the stranger is not the person you married.

The stranger phase is the phase when the recognition has gone, and the personality has gone, and the person you knew has gone, and the body is still here. The stranger phase is the phase of the long middle, in its deepest cut.

This chapter is about that phase. And about the work of being married to a stranger.

The arrival of the stranger.

The stranger does not arrive suddenly. The stranger arrives the way the disease arrives — slowly, in small steps, in the slope you have been watching for years. The stranger is the slope's final shape, in the body of the person you married.

The first signs that the stranger is arriving:

- The conversations stop. Not the conversations that you have to repeat. The conversations, period. The spouse no longer initiates. The spouse no longer responds. The spouse is present, but the

spouse is not engaging.

- The personality fades. The quirks, the mannerisms, the inside jokes, the way they used to laugh, the way they used to sigh, the way they used to look at you. These are gone. Or mostly gone. Or only there, in flashes, in the moments when the disease lifts for a second, and the person comes through, and then is gone again.
- The body becomes unfamiliar. The way they move. The way they hold their fork. The way they walk. The body is the same body, but the body is moving in ways you do not recognize.
- The look in the eyes changes. The eyes are still there. The eyes are still looking. But the look is not the look of recognition. The look is the look of someone who is seeing, but not seeing you.

The slow grief.

The grief of the stranger phase is a slow grief, not a sudden one. The sudden griefs were the diagnosis. The first forgetting. The day they forgot your name. These were the moments of acute loss.

The slow grief is the grief of the stranger. The slow grief is the grief of the person who is still here, but is no longer the person. The slow grief is the grief of the spouse who is still in the room, but is no longer the spouse. The slow grief is the grief that does not have a moment, but has a long, long duration.

The slow grief is the grief of the well spouse. The slow grief is the grief that comes, day by day, of being married to a stranger. The slow grief is the grief of holding the hand of a person who no longer knows your hand. The slow grief is the grief of being in the room, with a person who is in the room, but who is not, in any meaningful way, with you.

The slow grief is harder than the sudden griefs. The sudden griefs come, and you mourn them, and you go on. The slow grief is the mourning, all the time, for the duration. The slow grief is the work of being the spouse who remembers, when the spouse is no longer the spouse.

The work of being married to a stranger.

The work is not to pretend the stranger is the spouse. The work is not to talk to the stranger as if they were the spouse. The work is not to insist on the recognition, or the response, or the engagement.

The work is to be with the stranger, as the stranger. To be with the person who is here, not the person who used to be here. To be with the body, the breath, the presence. To be with the person who is in the room, in whatever form the person is now in.

The work is to find the new ways of being married, in the marriage that is, with the person who is. The work is to find the small touches. The work is to find the small moments. The work is to find the new forms of closeness, in the body, in the touch, in the presence.

The work is to grieve the marriage that was, in the same breath as the work of being in the marriage that is. The work is the holding of both, at the same time, the holding of the loss and the holding of the love.

The stranger is not the disease.

The stranger is the person. The stranger is the body, the breath, the presence, the person. The stranger is your spouse, in the form your spouse is now in. The stranger is not the disease. The disease took the recognition, the personality, the memory. The disease did not take the person.

The person is still in the body. The person is still in the breath. The person is still in the presence, in the room, in the moment, even when the moment is the only moment the person has.

The person is still the spouse. The person is still the one you married. The person is still the one you promised to love, in sickness and in health, for better or for worse. The person is still the one you are holding, in the stranger phase, in whatever form the holding can take.

The person is not the disease. The person is the spouse, in the form the spouse is now in.

The stranger in the bed.

The stranger in the bed is the most intimate version of the stranger. The stranger is in your bed, or in the bed in the facility, in the place where the marriage was made, and the marriage was held, and the marriage was the marriage, in the body, in the night.

The stranger in the bed is the spouse, in the form the spouse is now in. The stranger in the bed is the person you love, in the body of the person you love, in the place where the love was made.

The stranger in the bed is a presence. The stranger in the bed is a warmth. The stranger in the bed is a breath. The stranger in the bed is the marriage, in the body, in the night, in the form the marriage now takes.

The stranger in the bed is not the spouse who was. The spouse who was, is gone. The stranger in the bed is the spouse who is. The spouse who is, is the one you are holding, in the night, in the breath, in the body, in whatever form the holding can take.

The visits in the facility.

The visits in the facility are the stranger in another form. The stranger is in the facility, in the room, in the chair, in the bed. The stranger is the person you come to see, every day, or every week, or whenever you can.

The visits are the work. The visits are the act of being married, in the marriage that is, in the form the marriage now takes. The visits are the act of being present, in the presence that is no longer the presence of the spouse you remember, but is the presence of the spouse who is.

The visits are the small things. The hand on the hand. The kiss on the forehead. The story, told again, that the stranger no longer remembers, but that you still tell, because the story is the marriage, in the words. The song, played again, that the stranger no longer recognizes, but that you still play, because the song is the marriage, in the music.

The visits are the holding of the stranger, in whatever form the holding can take. The visits are the marriage, in the new shape.

The end of the stranger phase.

The stranger phase ends one of two ways. The stranger phase ends when the person dies. The stranger phase ends when you, the well spouse, find a way to be with the stranger that is not just endurance.

The first ending is not your choice. The first ending comes when the body gives out, and the person is gone, and the marriage is, at last, complete. The first ending is the ending of the long road, and the grief of the first ending is the grief of the entire long road, compressed.

The second ending is your choice. The second ending is the moment when you stop trying to make the

stranger be the spouse, and you start being with the stranger, as the stranger. The second ending is the moment when the marriage becomes the marriage that is, in the form the marriage now takes, without the resistance, without the fighting, without the wishing it were different.

The second ending is the work of acceptance. The second ending is the work of being present, in the presence that is. The second ending is the work of loving the stranger, in the form the stranger is now in.

The second ending is the marriage, in its final shape. The marriage that has survived the long road. The marriage that has survived the slope. The marriage that has survived the diagnosis, the placement, the stranger. The marriage that has survived, in the body, in the touch, in the presence, in whatever form the marriage now takes.

The second ending is the love that does not need recognition to exist. The love that has survived the stranger, and is still here, holding the hand, in the night, in the breath, in the body.

The love is the marriage. The love is the end of the stranger phase. The love is the work.

Chapter 14 at a Glance

Key takeaway: The stranger phase is the phase when the spouse is still in the room, but is no longer the person you married. The work is to be with the stranger, as the stranger, while honoring the spouse that was.

5 Things to Remember:

1. The stranger does not arrive suddenly. The stranger is the slope, in its final shape.

2. The slow grief is harder than the sudden griefs. The slow grief is the mourning, all the time, for the duration.

3. The stranger is not the disease. The stranger is the spouse, in the form the spouse is now in.

4. The visits in the facility are the marriage, in the new shape. The small touches are the language.

5. The love is the marriage. The love is the end of the stranger phase. The love is the work.

Your Scorecard: Rate 1-5:

- I have begun to be with the stranger, as the stranger, not as the spouse I remember.
- I am finding the small moments, the small touches, in the marriage that is.
- I am honoring the spouse that was, while being present with the spouse that is.
- I have begun to find the love that does not need recognition to exist.

If your total is 12 or lower, the most important thing is to be with the stranger today, as the stranger. Not as the spouse. Just as the person who is in the room. The being with is the love.

Key Terms to Know:

- The stranger phase: The phase in the long road when the spouse is still in the room, but is no longer the person you married.
- The slow grief: The grief of the stranger phase. The mourning, all the time, for the duration.
- The second ending: The moment when you stop trying to make the stranger be the spouse, and you start being with the stranger, as the stranger. The work of acceptance.

Reflection Questions:

1. What is the small thing that still arrives, in the stranger, that is still the spouse you remember?
2. What is the new form of presence, in the marriage that is, with the stranger who is?
3. What is the love, in the marriage that is, that does not need recognition to exist?

Action for This Week:

- Be with the stranger, today, as the stranger. Not as the spouse you remember. Just as the person who is in the room.
- Tell the story, one more time. The stranger may not remember. The story is the marriage, in the words.
- Touch the hand, one more time. The stranger may not know the hand. The touch is the marriage, in the body.

Chapter 15: The Grief That Has No Name

The grief of the spouse of someone with dementia is the grief that has no name. It is not the grief of widowhood, because the spouse is still alive. It is not the grief of divorce, because the marriage is still intact. It is not the grief of estrangement, because the spouse is still present. It is the grief of something that does not have a word in the language, because the situation itself is a situation that did not have a name, until recently.

This chapter is about that grief. About the grief that lives in the body of the well spouse, in the long road, in the years of the slow goodbye. About the grief that does not get a funeral, and does not get a sympathy card, and does not get the casseroles from the neighbors. About the grief that is, in some ways, the most difficult grief of all.

The grief that has no name.

Anticipatory grief. The grief that begins before the loss. The grief that is, in some ways, more difficult than the grief that comes after, because the grief that comes after has a finality, a moment, a place to put the loss. The anticipatory grief is the grief that is ongoing, that is daily, that is the work of the long road.

The anticipatory grief has several layers.

The first layer: the loss of the future. The grief of the future that was supposed to be yours. The retirement you were going to take together. The trips you were going to take. The grandchildren you were going to spoil. The quiet evenings of doing nothing together.

The future was a thing you were building, together, and the diagnosis took the future away, before the future arrived.

The second layer: the loss of the present. The grief of the present that is not the present you planned. The daily routines that have become the work of caregiving. The conversations that have become the work of repetition. The marriage that has become the work of being married, in the new form. The present is not the present you planned, and the present is the only present you have.

The third layer: the loss of the past. The grief of the past that is no longer the past you remember. The stories your spouse can no longer tell. The memories your spouse can no longer access. The past that was shared, that was the foundation of the marriage, that is now, in some essential way, gone. The past is the past, and the past is now only in you.

The fourth layer: the loss of the person. The grief of the person who is leaving, while they are still in the room. The grief of the personality that is fading. The grief of the recognition that is failing. The grief of the conversation that is becoming one-way. The grief of the spouse, in the body, but not in the mind, in the room, but not in the marriage.

The fifth layer: the loss of yourself. The grief of the self that you were, before the diagnosis. The self that was a spouse, a partner, an equal. The self that is now a caregiver, a case manager, a clinical presence. The grief of the self that is being lost, in the work of holding the marriage together, in the work of being the spouse who remembers.

The difficulty of the grief.

The anticipatory grief is difficult in ways that other griefs are not.

It is not socially recognized. The grief of widowhood is socially recognized. The widow gets the casseroles. The widow gets the sympathy. The widow gets the permission to grieve, openly, publicly, for as long as the grieving takes.

The spouse of someone with dementia does not get the casseroles. The spouse of someone with dementia does not get the sympathy. The spouse of someone with dementia does not get the permission to grieve, because the spouse is still alive. The spouse is expected to be the caregiver, the strong one, the one who is holding it together. The spouse is expected to be the spouse, in the way the spouse was before, even when the spouse is no longer the spouse.

It is not self-recognized. The spouse does not always recognize the grief as grief. The spouse may call it stress, or fatigue, or burnout, or depression. The spouse may not give themselves the permission to grieve, because the person is still alive, and the grieving feels like a betrayal, or a giving-up, or a wish for the end.

It is not time-limited. The grief of widowhood has a duration, however long. The grief ends, in time, in the way that grief ends, with the gradual return of the capacity for living. The anticipatory grief does not end, because the loss is ongoing. The anticipatory grief is the work of the long road, and the long road is long, and the long road does not end until the spouse dies.

It is not community-held. The grief of widowhood is held by the community. The community comes, with the casseroles, and the cards, and the listening. The

community holds the grief, for a while, until the widow can hold it alone.

The anticipatory grief is held alone. The caregiver is alone in the grief, in a way that the widow is not. The caregiver does not have the community. The caregiver has the work, and the spouse, and the long road. The caregiver does not have the casseroles, or the cards, or the permission.

The work of grieving.

The work of grieving the marriage that is, while the marriage is still happening, is the work of the long road. The work is not to skip the grief. The work is to honor the grief, and to let the grief have its place, in the body, in the life, in the long road.

The work of grieving has several parts.

Naming the grief. The first part is to name the grief, out loud, to yourself, and to someone who will hear it. I am grieving. I am grieving the future. I am grieving the present. I am grieving the past. I am grieving the person. I am grieving myself. I am grieving the marriage that was. I am grieving the marriage that is. The naming is the beginning. The naming is what allows the grief to be seen, and to be held, and to be worked with.

Honoring the grief. The second part is to honor the grief, in small ways, every day. The candle, lit on the day of the diagnosis, every year. The photo album, looked at, on the day of the wedding anniversary. The walk, taken, on the day of the first forgetting. The small things, done, in the small moments, to honor the grief, and to let the grief be part of the long road.

Finding the witnesses. The third part is to find the witnesses — the people who will hear the grief, and

hold it, and not try to fix it. The friend. The counselor. The support group. The other caregivers, in the Alzheimer's Association forum, in the local support group, in the online community. The witnesses are the people who will say, I hear you. I am here. You are not alone.

Letting the grief move. The fourth part is to let the grief move, in the body, in the life. The grief wants to move. The grief wants to cry, to scream, to write, to run, to be in the body, in the breath, in the work. The grief is the energy, and the energy wants to go somewhere. Let it go through the four doors (work, body, voice, wrong), and not through the wrong door, as much as you can.

Coming back to the marriage. The fifth part is to come back to the marriage, after the grieving, with the grief still in you, and the love still in you, and the work still to be done. The marriage is the holding of the grief and the love at the same time. The marriage is the holding of the loss and the presence at the same time. The marriage is the work, and the work is the marriage.

The permission to grieve.

The permission to grieve the spouse who is still alive is the permission that is hardest to give yourself. The permission feels like a betrayal. The permission feels like a wish for the end. The permission feels like a giving-up, or a failure, or a weakness.

The permission is none of these. The permission is the work. The permission is the recognition that the grief is real, and that the grief deserves to be honored, and that the honoring of the grief is the honoring of the marriage, and the honoring of the marriage is the work.

The permission is also the recognition that the grief, named and honored, is the grief that does not become the depression, or the bitterness, or the resentment. The grief, named and honored, is the grief that is held, and is worked with, and is part of the long road. The grief, unnamed and unheld, is the grief that becomes the thing that breaks the caregiver.

The permission to grieve is the permission to survive. The permission to grieve is the permission to be the spouse who remembers, for as long as the long road is. The permission to grieve is the permission to love, in the form the love now takes.

The grief that has no name.

The grief that has no name is the grief of the long road. The grief that has no name is the grief of the marriage that is, in the form the marriage now takes. The grief that has no name is the grief of the spouse who is still here, holding the hand, in the body, in the breath, in the night.

The grief that has no name is the grief that deserves to be honored, in the long road, in the small moments, in the candle on the day of the diagnosis, in the photo album on the day of the wedding anniversary, in the walk on the day of the first forgetting.

The grief that has no name is the grief that, in the end, is the love. The love is the grief. The grief is the love. They are the same thing, in the long road, in the marriage that is, in the form the marriage now takes.

The grief that has no name is the work. The work is the marriage. The marriage is the love. The love is the work.

Chapter 15 at a Glance

Key takeaway: The grief of the spouse of someone with dementia is the grief that has no name. It is the anticipatory grief, the daily grief, the grief of the long road. The grief is the work, and the work is the love.

5 Things to Remember:

1. The anticipatory grief has five layers: the future, the present, the past, the person, and yourself.
2. The grief is difficult because it is not socially recognized, not self-recognized, not time-limited, and not community-held.
3. The work of grieving: name it, honor it, find witnesses, let it move, come back to the marriage.
4. The permission to grieve is the permission to survive. The grief, named, does not become the depression.
5. The grief that has no name is the love. The love is the work. The work is the marriage.

Your Scorecard: Rate 1-5:

- I have named the grief, at least to myself.
- I have honored the grief, in at least one small way, this week.
- I have found at least one witness for the grief.
- I have begun to come back to the marriage, with the grief still in me.

If your total is 12 or lower, the most important thing is to name the grief. Say it out loud. To yourself, in a journal, to a friend, to a counselor. The naming is the beginning.

Key Terms to Know:

- Anticipatory grief: The grief that begins before the loss. The grief of the long road.

- The five layers: The future, the present, the past, the person, and yourself.
- The permission to grieve: The hardest permission to give yourself. The permission to grieve the spouse who is still alive.

Reflection Questions:

1. Which of the five layers of grief is the heaviest for you, right now?
2. What is the small thing you could do this week, to honor the grief?
3. Who is the one witness you can find, for the grief?

Action for This Week:

- Name the grief. Out loud. To yourself, in a journal, to a friend. I am grieving. I am grieving the [future, present, past, person, self]. I am grieving the marriage that was. I am grieving the marriage that is.
- Do one small thing to honor the grief. Light a candle. Look at the photo album. Take the walk. Write the letter that you do not send.
- Find one witness. Tell them: I need to tell you about the grief. I need you to hear it, and hold it, and not try to fix it. Can you do that?

Chapter 16: Surviving the Surviving

There is a moment, in the long road, when the end becomes visible. The moment when the slope, which has been a slope for years, becomes a cliff. The moment when the body, which has been declining slowly, begins to decline faster. The moment when the staff at the facility start using words like hospice, and comfort care, and the end is near.

This chapter is about that moment. And about the work of being the spouse who survives the surviving.

The end becomes visible.

The end becomes visible in small signs. The appetite goes. The sleep becomes most of the day. The mobility declines. The falls become more frequent. The infections come more often, and recover more slowly. The medications, which have been managing the symptoms, become managing the dying.

The end becomes visible in the body's slow withdrawal. The body is preparing to leave. The body is doing the work of leaving, in its own time, in its own way. The body is the last part to go, and the body goes slowly, and the body goes with a grace that no one talks about.

The work of being there.

The work, in this phase, is the work of being there. The work of being in the room, in the chair, in the bedside. The work of holding the hand, in the last days, in the last hours, in the last minutes.

The work of being there is the work of the marriage, in its final form. The work of being there is the holding

of the hand, in the body, in the breath, in the presence. The work of being there is the love that does not need recognition to exist, in the last hours, in the last breath.

The work of being there is also the work of the practical things. The funeral arrangements. The obituary. The notification of the family. The disposition of the body. The work of the practical things is the work of the grief that is coming, and the grief that is already here, and the grief that will be here, in the days and weeks and months after.

The work of the practical things is not the avoidance of the grief. The work of the practical things is the honoring of the grief, in the form the grief now takes. The work of the practical things is the work of saying goodbye, in the body, in the breath, in the practical details of the death.

The final days.

The final days are the work of being there, and the work of letting go. The two are not in conflict. The two are the same work, in two hands.

Being there is the hand that holds. Being there is the hand that touches the forehead, that strokes the hair, that holds the hand, in the bed, in the room, in the quiet. Being there is the act of presence, in the last days, in the form the presence can take.

Letting go is the hand that releases. Letting go is the act of saying, in the body, in the breath, in the words, It is okay to go. I will be okay. I will hold the marriage, in me, in the form the marriage now takes. I will hold the love, in me, in the form the love now takes. I will hold you, in me, in the form the holding can take.

Letting go is not giving up. Letting go is the final act of love, in the long road, in the form the love now takes. Letting go is the act of saying, I love you enough to let you go. I love you enough to let the marriage, in this form, end. I love you enough to let the long road end.

The final hours.

The final hours are the hours of the body, in its last work. The body is doing the work of leaving. The body is breathing, in the rhythm of the leaving. The body is present, in the form the presence now takes.

The final hours are the hours of the spouse who remembers, in the last work. The work of being in the room. The work of holding the hand. The work of saying the things that need to be said, before the saying is no longer possible.

The things that need to be said are simple. I love you. I will be okay. I will hold you, in me. Thank you. For the marriage. For the children. For the years. For the love. Thank you. I love you. I will be okay.

The things that need to be said are not the things that are said in the movies. The things that need to be said are the small things, the true things, the things that are said in the last hours, in the quiet, in the breath. The things that are said are the things that the spouse who remembers, in the last hours, in the breath, has been waiting to say.

The last breath.

The last breath is the breath that the body takes, in its last moment, in its last work. The last breath is the breath that comes, and goes, and does not come again.

The last breath is the moment of the death. The last breath is the moment when the slope ends. The last breath is the moment when the marriage, in this form, becomes the marriage, in the next form. The last breath is the moment when the spouse who is leaving, has left. The last breath is the moment when the spouse who is remembering, is now the spouse who is remembering alone.

The moment after.

The moment after the last breath is its own moment. The moment after is the moment of the body, still warm, in the bed, in the room. The moment after is the moment of the holding of the hand, in the last time. The moment after is the moment of the saying of the goodbye, in the last time.

The moment after is the moment when the grief, which has been the work, becomes the grief that is no longer anticipatory. The moment after is the moment when the grief becomes the grief of the death, and not the grief of the long road. The moment after is the moment when the grief becomes the grief that is socially recognized, in the casseroles, in the cards, in the visits, in the wake.

The moment after is the moment when the spouse who is remembering, is now the spouse who has remembered, and the spouse who is now remembering alone. The moment after is the moment when the marriage, in this form, is over, and the marriage, in the next form, is beginning.

The next form.

The next form of the marriage is the form of the spouse who remembers, alone. The next form of the marriage is the form of the spouse who carries the marriage, in the body, in the breath, in the memory, in

the form the marriage now takes.

The next form is the form of the widow, or widower. The next form is the form of the person who has survived the surviving. The next form is the form of the person who has held the marriage, in the long road, in the form the marriage now takes, and who is now holding the marriage, in the form the memory now takes.

The next form is not the end. The next form is the continuation. The next form is the marriage, in the form the marriage now takes, in the heart, in the memory, in the holding of the hand that is no longer the hand of the spouse, but is the hand of the spouse who remembers.

The surviving.

The surviving is the work of the next form. The surviving is the work of being the spouse who remembers, alone, in the days and weeks and months and years after the last breath. The surviving is the work of holding the marriage, in the memory, in the form the memory now takes.

The surviving is not a single event. The surviving is a long road, in its own right. The surviving is the road of the grief, in the form the grief now takes. The surviving is the road of the holding, in the form the holding now takes. The surviving is the road of the love, in the form the love now takes.

The surviving is the road that the spouse who remembers, walks, after the last breath, in the form the memory now takes. The surviving is the road that is the marriage, in the next form. The surviving is the road that is the love, in the next form. The surviving is the road that is the work, in the next form.

The surviving is the work. The work is the marriage. The marriage is the love. The love is the surviving.

The surviving is the work of the spouse who remembers, in the long road, in the form the road now takes.

The end of the book.

This is the end of the book, in one sense. The end of the practical work, of the long road, of the slope, of the marriage that was, in the form the marriage was.

The end of the book is not the end of the marriage. The end of the book is the end of the slope. The end of the book is the end of the form, in the form the marriage was. The end of the book is the beginning of the form, in the form the marriage now takes.

The end of the book is the beginning of the surviving. The beginning of the next form. The beginning of the holding, in the memory, in the form the memory now takes. The beginning of the love, in the form the love now takes.

The end of the book is the beginning of the work, in the next form. The work of the surviving. The work of the holding. The work of the love, in the form the love now takes.

The end of the book is the beginning of you. The beginning of the you who has survived the surviving. The beginning of the you who is the spouse who remembers, in the next form, in the form the memory now takes.

The end of the book is the beginning.

Chapter 16 at a Glance

Key takeaway: The end becomes visible, in the last days, in the last hours, in the last breath. The work of being there, and the work of letting go, are the same work, in two hands. The surviving is the next form of the marriage.

5 Things to Remember:

1. The end becomes visible in the body's slow withdrawal. The end is the slope, in its final shape.
2. The work of being there is the work of presence, in the last days, in the form the presence can take.
3. The work of letting go is the act of saying, It is okay to go. I will be okay. I will hold you, in me.
4. The last breath is the moment when the marriage, in this form, becomes the marriage, in the next form.
5. The surviving is the work. The work is the marriage. The marriage is the love. The love is the surviving.

Your Scorecard: Rate 1-5:

- I have begun to think about the final days, the final hours, the final breath.
- I have begun to say the things that need to be said, while the saying is still possible.
- I have begun to make the practical preparations, in the form the preparations can take.
- I have begun to imagine the next form, the surviving, the holding, in the form the memory now takes.

If your total is 12 or lower, the most important thing is to say the things that need to be said, while the saying is still possible. I love you. I will be okay. Thank you.

Key Terms to Know:

- Hospice: A philosophy of care that focuses on comfort and quality of life at the end of life, rather than on cure. Can be provided at home, in a facility, or in a hospice house.
- Comfort care: Medical care focused on relieving symptoms and maintaining comfort, rather than on treating the underlying disease. The standard of care at the end of life.
- The next form: The form the marriage takes, after the last breath, in the memory, in the holding.

Reflection Questions:

1. What is the thing you most want to say, before the saying is no longer possible?
2. What is the practical preparation you have been avoiding, that you can do this week?
3. What is the next form of the marriage, that you are beginning to imagine?

Action for This Week:

- Say the things that need to be said. I love you. I will be okay. Thank you. For the marriage. For the children. For the years.
- Make one practical preparation. The funeral arrangements. The obituary. The notification list. The will. The work of the practical things is the work of saying goodbye, in the form the goodbye can take.
- Imagine the next form. The surviving. The holding. The love, in the form the love now takes. The imagination is the work.

Epilogue: The Marriage That Lasts

There is a marriage that lasts.

It is not the marriage that was, in the form the marriage was. The marriage that was, in the form the marriage was, ended, the day the slope began.

It is not the marriage that is, in the form the marriage is now. The marriage that is, in the form the marriage is now, will end, the day the last breath comes.

The marriage that lasts is the marriage that is, in the form the marriage was, and the form the marriage is, and the form the marriage will be, in the memory, in the holding, in the love, in the form the love now takes.

The marriage that lasts is the marriage that the spouse who remembers, carries, in the body, in the breath, in the long road, in the form the road now takes. The marriage that lasts is the marriage that the spouse who remembers, holds, in the night, in the quiet, in the moments when the memory is the only thing that is left of the marriage that was.

The marriage that lasts is not a happy marriage. The marriage that lasts is not a sad marriage. The marriage that lasts is a marriage that has survived the long road, in the form the road now takes, and is now surviving, in the form the surviving now takes.

The marriage that lasts is the marriage that is the love. The love is the marriage. The marriage is the love. The love is the work. The work is the marriage. The marriage is the surviving. The surviving is the love.

The marriage that lasts is the marriage that is, in you, in the form the marriage now takes.

If you are reading this, and you are the spouse who is in the long road, right now, then this book is for you. If you are the spouse who is at the bedside, in the last hours, then this book is for you. If you are the spouse who is in the surviving, in the days and weeks and months after, then this book is for you.

If you are the adult child, reading this for a parent who is in the long road, then this book is for you. If you are the friend, reading this for someone who is in the long road, then this book is for you. If you are the spouse who has not yet begun, but who will begin, someday, in the long road, then this book is for you.

The long road is a road that many of us will walk. The long road is a road that few of us will walk well. The long road is a road that no one can walk for you. The long road is a road that the spouse who remembers, walks, in the form the road now takes.

The long road is a road that is, in the end, the marriage. The marriage is, in the end, the love. The love is, in the end, the surviving. The surviving is, in the end, the work.

The work is the marriage. The marriage is the love. The love is the surviving. The surviving is the work.

Welcome to the long road. Welcome to the marriage. Welcome to the love.

Welcome to the work.

— S. Shharma

About the Author

S. Shharma is the author of The Care Collection — books for couples in the second half of life, written by a man who lived the silence and survived it.

He lost his wife of many years to cancer. He was her primary caregiver through every stage of her illness. He held her hand in the hospital. He sat in the waiting room at 3 a.m. He learned the silence of a marriage the way you learn a language you never wanted to speak.

He is also a businessman, an M.Sc., an MBA, and an APSCM from the Indian Institute of Management (IIM) Calcutta, with thirty years in corporate management. But these are not credentials you need to trust him. The credentials that matter are the ones in the books.

The Care Collection is not about how to fix a marriage. The Care Collection is about how to live one — even when the marriage is quiet, even when one of you is sick, even when one of you is gone.

He lives in India. He writes from the perspective of someone who has been on every side of a marriage — the partner, the caregiver, and the one left behind.

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